

Department of Health Care Policy and Financing

Medical Services Board

MEDICAL ASSISTANCE – SECTION 8.900 OAP, PRIMARY CARE FUND, DENTAL HEALTH CARE

10 CCR 2505-10 8.900

[Editor's Notes follow the text of the rules at the end of this CCR Document.]

8.900 [Repealed effective 7/1/2025]

8.920 Hospital Discounted Care

The Health Care Billing for Indigent Patients Act of 2021, C.R.S. § 25.5-3-501, et. seq., referred to as Hospital Discounted Care, establishes the maximum rate a Health Care Facility and Licensed Health Care Professional may bill low-income patients for Discounted Care provided in the hospital, requires written description of patient's rights, establishes patient appeals and complaint processes, and imposes requirements on hospitals before assigning or selling patient debt to a medical creditor or before pursuing collection action. Senate Bill 24-116 added an increase in payment plan amounts for Health Care Facilities who bill for their own professionals, requires professionals who bill separately from the Health Care Facility to report their own data to the Department, and excludes primary care provided at rural or frontier clinics who have an approved sliding fee scale.

8.921 DEFINITIONS

- A. Billing Statement means any patient-facing communication, whether electronic or in writing, that specifies an amount due for services and instructions for making payment.
- B. Children's Basic Health Plan or the Child Health Plan Plus (CHP+) means the Children's Basic Health Plan as defined in Title 25.5, Article 8, C.R.S.
- C. Department means the Department of Health Care Policy and Financing established pursuant to section 25.5-1-104, C.R.S.
- D. Discounted Care means the amount a Provider may charge a Qualified Patient for Medically Necessary Health Care Services rendered.
- E. Emergency Medicaid means short-term Medicaid coverage for eligible people who do not meet immigration or citizenship requirements for Medicaid and need treatment for life- and/or limb-threatening emergencies.

- F. Emergency Hospital Services means treatment for conditions of an acute, severe nature which are life, limb, or disability threats requiring immediate attention, where any delay in treatment would, in the judgment of the responsible physician, threaten life or loss of function of a patient or viable fetus.
- G. Federal Poverty Guidelines or FPG means a measure of income level issued annually by the United States Department of Health and Human Services. For Hospital Discounted Care, the FPG is updated annually every April 1.
- H. Health Care Facility means a hospital licensed as a general hospital pursuant to Title 25, Article 3, Part 1, C.R.S., a hospital established pursuant to section 23-21-503, C.R.S. or section 25-29-103, C.R.S., any freestanding emergency department licensed pursuant to section 25-1.5-114, C.R.S., or any outpatient health care facility that is licensed as an on-campus department or service of a hospital or that is listed as an off-campus location under a hospital's license. Health Care Facility does not include a federally qualified health center as defined in the federal "Social Security Act", 42 U.S.C. sec. 1395x(aa)(4), or a student-learning medical or dental clinic that is established for the purpose of student learning, offering Discounted Care as part of a program of student learning that is physically situated within a health sciences school, Health Care Facility does not apply to primary care services provided in a clinic located in a designated rural or frontier county that offers a sliding-fee scale equal to the Medicare rural health clinic all inclusive rate payment established in accordance with 42 U.S.C. 1395l(f)(3)(B) or payment rate that is lower than usual and customary charges and considers a patient's household size and income size as approved by the Department.
- I. Health Care Services has the same meaning as set forth in section 10-16-102(33), C.R.S.
- J. Impermissible Extraordinary Collection Action means initiating foreclosure on an individual's primary residence or homestead, including a mobile home, as defined in section 38-12-201.5(5), C.R.S.
- K. Inpatient Hospital Service has the same meaning as set forth in 42 C.F.R. § 440.10. 42 C.F.R. § 440.10 (2024) is hereby incorporated by reference into this rule. Such incorporation, however, excludes later amendments to or editions of the referenced material. This regulation is available for public inspection at the Department of Health Care Policy and Financing, 303 E. 17th Ave, Denver, CO 80203. Pursuant to C.R.S § 24-4-410(12.5)(V)(b), the Department shall provide certified copies of the material incorporated at cost upon request or shall provide the requestor with information on how to obtain a certified copy of the material incorporated by reference from the agency of the United States, this state, another state, or the organization or association originally issuing the code, standard, guideline or rule.

- L. Licensed Health Care Professional or Professional means any health care professional who is registered, certified, or licensed pursuant to Title 12, C.R.S. or who provides services under the supervision of a health care professional who is registered, certified, or licensed pursuant to Title 12, C.R.S. and who provides Health Care Services in a Health Care Facility.
- M. Medicaid means the Colorado Medical Assistance Act set forth in Title 25.5, Articles 4, 5, and 6, C.R.S.
- N. Medical Creditor means any entity that attempts to collect on a medical debt, including a Provider or Provider's billing office, a collection agency as defined in section 5-16-103(3), a debt buyer as defined in section 5-16-103(8.5), C.R.S. and a debt collector as defined in 15 U.S.C. sec. 1692a(6).
- O. Outpatient Hospital Service has the same meaning as set forth in 42 C.F.R. § 440.20. 42 C.F.R. § 440.20 (2024) is hereby incorporated by reference into this rule. Such incorporation, however, excludes later amendments to or editions of the referenced material. This regulation is available for public inspection at the Department of Health Care Policy and Financing, 303 E. 17th Ave, Denver, CO 80203. Pursuant to C.R.S § 24-4-410(12.5)(V)(b), the Department shall provide certified copies of the material incorporated at cost upon request or shall provide the requestor with information on how to obtain a certified copy of the material incorporated by reference from the agency of the United States, this state, another state, or the organization or association originally issuing the code, standard, guideline or rule.
- P. Patient Contact Best Efforts means the process of communication efforts completed by the Provider to contact a patient. This includes phone calls, SMS messages, emails, and portal messages.
- Q. Permissible Extraordinary Collection Action means an action other than an Impermissible Extraordinary Collection Action that requires a legal or judicial process, including but not limited to placing a lien on an individual's real property, attaching or seizing an individual's bank account or any other personal property, or garnishing an individual's wages. A Permissible Extraordinary Collection Action does not include the attachment of a hospital lien pursuant to section 38-27-101, C.R.S.
- R. Provider means any Health Care Facility or Licensed Health Care Professional subject to Title 25.5, Article 3, Part 5, C.R.S.
- S. Qualified Patient means an individual who attests to residing in Colorado, whose household income is not more than two hundred fifty percent of the Federal Poverty Guidelines and who received an Inpatient Hospital Service or Outpatient Hospital Service at a Health Care Facility.

- T. Screen or Screening means a process identified in rule by the Department whereby Health Care Facilities assess a patient's circumstances related to eligibility criteria and determine whether the patient is likely to qualify for public health care coverage or Discounted Care, inform the patient of the Health Care Facility's determination, and provide information to the patient about how the patient can enroll in public health care coverage.
- U. SMS means short messaging service messages, commonly referred to as text messages.
- V. Uninsured means an uninsured individual, as defined in section 10-22-113(5)(d), C.R.S.

8.922 SCREENING AND APPLICATION

- A. Screening, Application, and Determination Notice
 - 1. Beginning September 1, 2022, using the single uniform application developed and distributed by the Department, a Health Care Facility shall screen each uninsured patient and any insured patients who request to be screened for:
 - a. Public health insurance programs including but not limited to Medicare, Medicaid, Emergency Medicaid, and the Children's Basic Health Plan.
 - b. Discounted Care, as described in section 25.5-3-503, C.R.S.
 - 2. Uninsured Patients
 - a. Health Care Facilities must complete the screening process using the uniform application within 45 days from the uninsured patient's date of service or date of discharge, whichever is later.
 - b. The screening process consists of completing the first page of the uniform application using self-attested information provided by the patient or their guardian.

- c. If the self-attested screening process results in a determination that the patient may be eligible for Discounted Care, then, at the time of the screening, the Health Care Facility must provide the patient or their guardian with a list of information and documents required to complete the application process. The patient is permitted 45 days to provide the documentation required to complete the application. When all necessary documentation has been received from the patient, the Health Care Facility must determine the patient's eligibility for Discounted Care and send written notice of the determination to the patient or guardian within 21 days.
 - d. If the self-attested screening process results in a determination that a patient likely is ineligible for Discounted Care, the patient must be informed that the screening results are not an official determination and that they have the right to complete the application and receive an official determination of eligibility for Discounted Care if they choose. If the patient requests to complete the application process for Discounted Care, the Health Care Facility must complete the application process and provide an official determination of eligibility for Discounted Care.
 - e. If the self-attested screening process results in a determination that the patient may be eligible for one or more public health coverage options, the Health Care Facility must inform the patient of those options and provide information on how the patient may apply for them, including any application deadlines the patient should be aware of.
- 3. Insured Patients
 - a. If the insured patient or their guardian requests to be screened for public health insurance programs and Discounted Care, Health Care Facilities must screen insured patients within 45 days of their date of service or date of discharge, or within 45 days of the date of their first bill after their insurance adjustment, whichever is later.
 - b. The request to be screened may be made in person, by telephone, email, or by using the portal, if available. Health Care Facilities must contact the insured patient or their guardian to schedule the screening within three business days after receiving the insured patient's request.

- c. Patients believed to have health insurance coverage when services were rendered and who are subsequently determined to be uninsured on their date of service are considered Uninsured. Within 45 days from the date of the notification that the patient was not insured on the date of service, the Health Care Facility must complete the screening.
- 4. Presumptive Eligibility for State Medical Assistance Program
 - a. Hospitals may not complete a presumptive eligibility screening for a patient in lieu of completing a screening as required in this section.
 - b. A hospital that is not a Qualified Hospital Presumptive Eligibility Site as defined in 10 CCR 2505-10 Section 8.100.1 may not use presumptive eligibility to deny discounted care to any patient.
- 5. Health Care Facility Determination Notice
 - a. The Health Care Facility must provide the patient written notice of the determination within 21 days of receiving all required documentation to complete the patient's application for Discounted Care. A copy of the determination must be sent to any and all applicable Licensed Health Care Professionals.
 - b. The determination shall be written in plain language and in the patient or their guardian's preferred language.
 - c. If a Health Care Facility fails to issue written notice of the determination to the patient within 21 days of receiving all required documentation to complete the patient's application, the patient may file an appeal. If the appeal is filed within 60 calendar days of the patient submitting all required documentation, the Health Care Facility must review the appeal and respond to the patient or their guardian and the Department within 15 calendar days of the date of the appeal.
 - d. For patients determined to be eligible for Discounted Care, the determination notice must include but is not limited to:
 - 1. The programs and discounts for which the patient was determined likely eligible for, including but not limited to Medicaid, Emergency Medicaid, CHP+, Medicare, and Hospital Discounted Care, and the availability of subsidies through Connect for Health Colorado. This must also include where to find additional information and how to apply for each program the patient was determined potentially eligible for.

- i. If the patient appears likely eligible for a program, and there is a deadline by which the patient must apply for that program for their services to be covered, that date must be included in the determination notice.
 2. The dates for which the Discounted Care determination is valid.
 3. The household size and income used to determine eligibility and the household calculated FPG.
 4. The patient's monthly installment amounts calculated on their gross household income pursuant to Section 8.923.A.2.
 5. Information on how to file a complaint and how to file an appeal with the Health Care Facility and the Department.
- e. The determination notice for patients determined not eligible for Discounted Care must include but is not limited to:
 1. The basis for denial of Discounted Care.
 2. The programs and discounts for which the patient was determined likely eligible for, including but not limited to Medicaid, Emergency Medicaid, CHP+, Medicare, and the availability of subsidies through Connect for Health Colorado. This must also include where to find additional information and how to apply for each program the patient was determined potentially eligible for.
 - i. If the patient appears likely eligible for a program, and there is a deadline by which the patient must apply to that program for their services to be covered, that date must be included in the determination notice.
 3. The service date the Discounted Care denial covers and an explanation that the household may qualify for coverage of future services if there is a change in household size or income.
 4. The household size and income used to determine eligibility and the household calculated FPG.
 5. Information on how to file a complaint and how to file an appeal with the Health Care Facility and the Department.

6. A Health Care Facility is no longer obligated to screen an uninsured patient for past dates of service if the patient or their guardian signs the decline screening form developed by the Department that notes those specific dates of service or a past date range that includes those specific dates of service except when a patient or guardian who opted out of screening subsequently requests to complete the screening, if the subsequent request is made prior to starting Permissible Extraordinary Collections Actions.
 - a. The Health Care Facility must keep on file a decline screening form signed by the patient, or their guardian until June 30 of the seventh state fiscal year after the patient's date of service or date of discharge, whichever is later.
7. For patients who are discharged without being screened or signing the decline screening form, the Health Care Facility must attempt to contact the patient by at least one method of contact that the patient indicates is their preferred method, which can include phone call, SMS message, email, and portal message at least once a month for six months after the patient's date of discharge with the first contact sent prior to the expiration of 45 days after screening. The Health Care Facility may commence billing 46 days after the patient's date of service or date of discharge, whichever is later. If the patient requests that the Health Care Facility cease contacting them by phone, SMS message, or email, the provider may consider those requirements as fulfilled. The Health Care Facility must document the patient's request and maintain the request as part of the patient record.
8. If a Health Care Facility has attempted to contact the patient in accordance with Patient Contact Best Efforts, and the patient does not respond within 182 days of their date of service or date of discharge, whichever is later, the Facility may conclude that the patient has made an informed decision to decline screening. Patient Contact Best Efforts, at a minimum, must include:
 - a. Notice that the failure to respond may result in the loss of their right to be screened for cost saving options.
 - b. Calling any phone numbers provided by the patient and leaving voice messages with allowable information under the Health Insurance Portability and Accountability Act as defined at 45 C.F.R. sec. 164.502 and the Telephone Consumer Protection Act as defined at 47 U.S.C. sec. 227 if the calls are unanswered,

1. 45 C.F.R. § 164.502 (2024) is hereby incorporated by reference into this rule. Such incorporation, however, excludes later amendments to or editions of the referenced material. This regulation is available for public inspection at the Department of Health Care Policy and Financing, 303 E. 17th Ave, Denver, CO 80203. Pursuant to C.R.S § 24-4-410(12.5)(V)(b), the Department shall provide certified copies of the material incorporated at cost upon request or shall provide the requestor with information on how to obtain a certified copy of the material incorporated by reference from the agency of the United States, this state, another state, or the organization or association originally issuing the code, standard, guideline or rule.
 - c. SMS messages to any of the patient's phone numbers identified by the patient as a mobile number if the Health Care Facility has the ability to send SMS messages,
 - d. Sending emails to any email address provided by the patient, and
 - e. Sending messages through any appropriate patient portal.
 9. If a patient does not indicate their preferred method of contact, the Provider shall contact patients in accordance with their internal patient communication policies. Documentation of the communication attempts for patients must be kept in their patient records and the communication policy must be kept on file until the June 30 of the seventh state fiscal year past the patient's date of service.
 10. Documentation of the attempts to contact the patient or guardian to complete the screening must be maintained as part of the patient record. This may include call logs, message logs, copies of sent emails, portal messages sent, and copies of bills.
 11. Providers shall maintain all Discounted Care-related records, including but not limited to, documentation to support screenings and determinations, service data including dates of service for Qualified Patients and services provided to them on those dates, and expenditures until June 30 of the seventh state fiscal year following the creation of the documentation.
- B. Patients**
1. Any patient or patient's guardian aged 18 and older may apply to receive Discounted Care.
 2. The decision regarding eligibility for Discounted Care applies to both the patient and the members of the patient's household.

3. If a patient is deceased, the personal representative of the estate or a family member may complete the screening and application on behalf of the patient.
4. The application to receive Discounted Care shall include the names, birth dates, and relationship to the patient of all members of the patient's household who are included on the application.
 - a. A patient must include their spouse or civil union partner in their household for the application.
 - b. Any additional person living at the same address as the patient may also be included in the household.
 - c. A patient may include household members who live in other states or countries if the patient attests to the fact that they provide at least 50% of the household member's support.
5. A minor shall not be screened separately from his or her parents or guardians unless they are emancipated or there exists a special circumstance. A minor is an individual under the age of 18.

C. Household Income

1. Using the information submitted by a patient or patient's guardian, the Health Care Facility shall determine whether the patient meets all requirements to receive Discounted Care. Health Care Facilities must follow the income counting methodology determined by the Department. Health Care Facilities shall determine Qualified Patient financial eligibility based on income from each household member 18 and older and household size. The Health Care Facility may not consider assets in determining eligibility.
2. Eligibility shall be determined at the time of application, unless required documentation is not available, in which case the patient or patient's guardian will be notified of the missing documentation within three business days after receipt of the application. An eligibility determination shall be made within 21 calendar days after the application is complete.
3. Patients may establish household income by providing documents that satisfy documentation guidelines established by the Department. Acceptable forms of documentation may include but is not limited to pay stubs, employer letter, tax returns, and business financial statements. The Health Care Facility may not require more than the minimum amount of documentation to substantiate declared income.

- a. Patients who are experiencing homelessness are exempt from the documentation requirements related to establishing income and may self-attest to their household income.

8.923 HEALTH CARE SERVICE DISCOUNTS

- A. Beginning September 1, 2022, if a patient screened pursuant to Section 8.922 is determined to be a Qualified Patient, a Health Care Facility and a Licensed Health Care Professional shall for Emergency Hospital and other Health Care Services:
 1. Limit the amounts billed for Health Care Services to no more than the rate established in Department rule pursuant to Section 8.928.
 2. Enter into a payment plan with the Qualified Patient in which the Qualified Patient pays for care in monthly installments. For services provided by a Health Care Facility, monthly installments shall not exceed four percent of the patient's gross monthly household income on a bill from a Health Care Facility that contains only facility charges and shall not exceed six percent of the patient's gross monthly household income on a bill from a Health Care Facility containing both facility and Licensed Health Care Professional charges. For services provided by each Licensed Health Care Professional who bills separately from the Health Care Facility, monthly installments shall not exceed two percent of the patient's gross monthly household income; and
 3. After a cumulative thirty-six months of payments, the Health Care Facility shall treat the Qualified Patient's bill as paid in full and must permanently cease collection activities on any balance that remains unpaid.
 4. Providers shall not suggest or require that patients obtain loans that include fees, interest, or payment plans that exceed 36 payments to pay for services in lieu of setting up a payment plan directly with the Health Care Facility or Licensed Health Care Professional.
 - a. If a patient defaults on a loan from the Provider, the same rules apply related to any collection actions taken by the Provider as apply for payment plans under this section. If a patient defaults on a loan from the Provider, the same rules apply related to any collection actions taken by the Provider as apply for payment plans under this section..
- B. A Health Care Facility shall not:
 1. Deny Discounted Care on the basis that the patient has not applied for any public benefits program, unless during the initial screening the patient is determined to be presumptively eligible for the state medical assistance program; or

2. Adopt or maintain any policies that result in the denial of admission or treatment of a patient because the patient may qualify for Discounted Care.

8.924 PATIENT RIGHTS

- A. Beginning September 1, 2022, a Health Care Facility shall make available to the public and to each patient information developed by the Department about patient's rights pursuant to Part 5 of Article 3 of Title 25.5 C.R.S. (2021) and the uniform application developed by the Department pursuant to section 25.5-3-505(2)(i), C.R.S.
- B. At a minimum, the Health Care Facility shall:
 1. Post the information in all languages spoken by ten percent or more of the population in any Colorado county conspicuously on the Health Care Facility's website, including a link to the information on the Health Care Facility's main landing page;
 2. Make the information available in patient waiting areas;
 3. Make the information available to each patient, or the patient's legal guardian, before the patient is discharged from the Health Care Facility, verbally or in writing in the patient's or legal guardian's preferred language, which may include using professional interpretation and/or translation services; and
 4. Inform each patient on the patient's Billing Statement of the patient's rights pursuant to Part 5 of Article 3 of Title 25.5, C.R.S. (2021) including the right to apply for Discounted Care, and provide the website, email address, and telephone number where the information may be obtained in the patient's preferred language.
- C. Providers shall not present the patient's rights in a format that differs from the format in which the material is distributed by the Department without Department approval.
 1. Providers may not make any part of the patient's rights information part of a footnote or use any other format that may minimize its importance.

8.925 REPORTING REQUIREMENTS

- A. Beginning September 1, 2023 for Health Care Facilities and beginning September 1, 2025 for Licensed Health Care Professionals, and each September 1 thereafter, each Health Care Facility and Licensed Health Care Professional shall report to the Department data that the Department determines is necessary to evaluate compliance across race, ethnicity, age, and primary-language-spoken patient groups with the screening, Discounted Care, payment plan, and collections practices required by Title 25.5, Article 3, Part 5, C.R.S. . The Department shall distribute a compliance data reporting template to each Health Care Facility.
1. If a Health Care Facility or Licensed Health Care Professional is not capable of disaggregating the required data by race, ethnicity, age, and primary language spoken, the Health Care Facility or Licensed Health Care Professional shall report to the Department the steps the Health Care Facility or Licensed Health Care Professional is taking to improve race, ethnicity, age, and primary language spoken data collection and the date by which the facility or Licensed Health Care Professional will be able to disaggregate the reported data.
- B. Beginning September 1, 2023 for Health Care Facilities and beginning September 1, 2025 for Licensed Health Care Professionals, and each September 1 thereafter, each Health Care Facility and Licensed Health Care Professional shall submit Discounted Care utilization and charge data in a format and timeline determined by the Department.

8.926 COLLECTIONS

- A. Beginning September 1, 2022, before assigning or selling patient debt to a collection agency or a debt buyer, or before pursuing, either directly or indirectly, any Permissible Extraordinary Collection Action:
1. A Health Care Facility shall meet the screening requirements in Section 8.922;
 2. A Provider shall provide Discounted Care to a Qualified Patient pursuant to Section 8.920;
 3. A Provider shall provide a plain language explanation of the health care services and fees and notify the patient or their guardian of potential collection actions in their preferred language on the timeline developed by the Department; and

4. A Provider shall bill any third-party payer that is responsible for providing health care coverage to the patient. If a Licensed Health Care Professional is an out-of-network provider under a Qualified Patient's health insurance plan, the Licensed Health Care Professional and health insurance carrier shall comply with the out-of-network billing requirements described in sections 10-16-704 (3) and 12-30-113, C.R.S.
- B. A Health Care Facility must complete the Patient Contact Best Efforts in their attempts to contact a patient who has not signed a Decline Screening Form or who has not been screened as described in Section 8.922 prior to starting Permissible Extraordinary Collections Actions.
- C. Documentation of Patient Contact Best Efforts communication attempts with the patient as outlined in Section 8.922 satisfies the screening requirements for Health Care Facilities.
- D. For a Qualified Patient with an established payment plan, Permissible Extraordinary Collections Actions may not be started until the patient has failed to remit three consecutive payments and has not communicated with the Provider asking for a deferment or to be redetermined prior to or during those three months of missed payments. Providers must notify Qualified Patients with established payment plans at least 30 days prior to the commencement of Permissible Extraordinary Collections Actions.
- E. Providers shall not commence collection proceedings against a patient for any amount in excess of the rates established at Section 8.923.A.2, and must reduce the amount owed by the amount of any payments received from the patient or a third-party payer.

8.927 APPEALS AND COMPLAINTS

- A. If a patient is determined ineligible for Discounted Care after the uniform application has been completed, the patient may appeal the decision as follows:
 1. No later than 30 calendar days from the date on the Health Care Facility's eligibility determination letter, the patient or their guardian may submit an appeal in writing via U.S. Mail, email, or patient portal message if available to the Health Care Facility that made the determination.
 2. Within 15 calendar days from the date of the appeal, the Health Care Facility shall complete a redetermination of eligibility and respond to the patient or guardian and the Department.
 3. If the Health Care Facility upholds its initial eligibility determination, the patient or guardian may proceed to the next step of the appeals process as described in Section 8.927.A.4.

4. No later than 15 calendar days from the date of the Health Care Facility's initial appeal decision, the patient shall submit a written appeal to the Department. Email submissions must be addressed to hcpf_HospDiscountCare@state.co.us. Letters must be mailed to:

Department of Health Care Policy and Financing
Attention: State Programs Unit, Special Financing Division
c/o Hospital Discounted Care
303 E. 17th Avenue Suite 1100
Denver, CO 80203
 5. Within 15 calendar days from date of receipt of the appeal, the Department shall issue a final determination letter to both the patient and the Health Care Facility. If the Department deems that the redetermination was inaccurate, the Health Care Facility must resend a determination letter to the patient and the Department stating the patient is/was eligible for Discounted Care on the date of service.
- B. A patient or guardian who believes a Health Care Facility has improperly calculated a payment plan based on inaccurate income information may appeal the payment plan offered by the Facility to the Department using the process described in Section 8.927.A.
- C. The Department shall maintain records of all appeals and its final determinations for each Health Care Facility. If the Department determines a Health Care Facility has a repeated pattern of errors in patient eligibility determinations, the Department will require the Health Care Facility to attend training with the Department. The Health Care Facility may be subject to random application checks for 12 months following the training to ensure that the errors have been corrected.
- D. Patients and their guardians may file complaints against Providers directly with the Department. Patients are not required to file a complaint with the Provider prior to filing a complaint with the Department.
1. Patients may submit complaints via U.S. Mail, email, or phone as follows:

Phone: 303-866-2580
Email: hcpf_HospDiscountCare@state.co.us

U.S. Mail: Department of Health Care Policy and Financing
Attention: State Programs Unit, Special Financing Division
c/o Hospital Discounted Care
303 E. 17th Avenue Suite 1100
Denver, CO 80203
 2. The Department shall review complaints within 30 calendar days of receipt.

3. The Department shall maintain records of all complaints for each Provider. If the Department determines there is a repeated pattern in the complaints filed against the Provider, the Provider may be subject to a corrective action plan.
 - a. Providers will have 90 days to submit a corrective action plan. Extensions may be made at the Department's discretion up to no more than 120 days.

8.928 HOSPITAL DISCOUNTED CARE PROGRAM STRUCTURE

8.928.1 REVIEW OF PROVIDERS FOR NONCOMPLIANCE

- A. The Department will periodically review Providers to ensure compliance with Part 5 of Article 3 of Title 25.5, C.R.S. (2024) and these rules. If the Department finds that a Provider is not in compliance with these rules, the Department will notify the Provider.
- B. The Provider will have 90 days to file a corrective action plan with the Department that must include measures to inform impacted patients about the noncompliance and provide financial corrections consistent with these rules.
 1. At the Department's discretion, a Provider may be permitted up to 120 days to submit a corrective action plan upon request.
 2. The Department may require a Provider that is not in compliance with Title 25.5, Article 3, Part 5, C.R.S. or these rules to develop and operate under a corrective action plan until the Department determines the Provider is in compliance.
- C. If a Provider's noncompliance with these rules is determined by the Department to be knowing or willful or there is a repeated pattern of noncompliance, the Department may fine the Provider no more than \$5,000. If the Provider fails to take corrective action or fails to file a corrective action plan with the Department pursuant to this section, the Department may fine the Provider no more than \$5,000 per week until the Provider takes corrective action. The Department will consider the size of the Health Care Facility and the seriousness of the violation in setting the fine amount.
- D. The Department will make the information reported pursuant to this section and any corrective action plans for which fines were imposed pursuant to this section available to the public and shall annually report the information as part of its presentation to its committees of reference at a hearing held pursuant to section 2-7-203 (2)(a), C.R.S. of the "State Measurement for Accountable, Responsive, and Transparent (SMART) Government Act."

- E. For audit purposes, Providers shall maintain all Discounted Care related records, including but not limited to, documentation to support screenings and determinations, service data including dates of service for Qualified Patients and services provided to them on those dates, and expenditures until June 30 of the seventh state fiscal year following the screening or determination.

8.928.2 RATES

The Department will annually establish rates for Discounted Care. The rates will approximate and not be less than one hundred percent of the Medicare rate or one hundred percent of the Medicaid rate, whichever is greater. The Department will publicly post the established rates on the Department's website pursuant to section 25.5-3-505(2)(j), C.R.S.

8.928.3 ADVISORY COMMITTEE

- A. Committee Makeup. The Department will create a Hospital Discounted Care Advisory Committee, effective July 1, 2025. The Executive Director of the Department will appoint 11 members to the Hospital Discounted Care Advisory Committee. Committee members will include:
1. A member representing the Department;
 2. Three members who are health care consumers, of whom no more than two members may be employed by a health care consumer advocacy organization;
 3. A member who is a representative of a safety net hospital for which the percent of Medicaid-eligible inpatient days relative to the hospital's total inpatient days is equal to or greater than one standard deviation above the mean;
 4. A member who is a representative of a hospital in a rural area;
 5. A member who is a representative of a hospital in an urban area;
 6. A member who is a representative of a statewide organization of hospitals;
 7. A member who is a representative of licensed health care professionals who provide services to patients in a hospital setting;
 8. A member who is a representative of an organization of Colorado community health centers or a representative of a Colorado community health center, as defined in 42 U.S.C. § 254b;

9. A member who is a representative of an organization of safety net health providers or a safety net health provider that is not a community health center.

Members shall serve without compensation or reimbursement of expenses. The Executive Director of the Department will designate a member to serve as chair of the committee and the appointment will be valid until the seat is vacated, the chair steps down, or a new chair is selected by the Executive Director. The council shall convene at least twice every state fiscal year according to a schedule set by the chair. Members of the council shall serve three-year terms. Of the members initially appointed to the advisory committee, the Executive Director will appoint six members for two-year terms and five members for three-year terms. In the event of a vacancy on the advisory committee, the Executive Director will appoint a successor to fill the unexpired portion of the term for the member.

B. Committee Duties. The advisory committee shall:

1. Advise the Department on the operations and policies for Hospital Discounted Care, and
2. Make recommendations to the Medical Services Board regarding rules for Hospital Discounted Care.

8.930 [Repealed effective 8/12/2011.]

8.940 OLD AGE PENSION HEALTH CARE PROGRAM

8.941 EXTENT AND LIMITATIONS OF MEDICAL CARE

8.941.1 GENERAL DESCRIPTION - OLD AGE PENSION HEALTH CARE PROGRAM

In accordance with the Constitution of Colorado, Article XXIV, Section 7, and the Colorado Public Assistance Act, an Old Age Pension Health Care Program is established to provide necessary medical care for the Old Age Pension (OAP) recipients who do not qualify for Medicaid under Title XIX of the Social Security Act and Colorado statutes. The State Department is designated as the single State agency to administer the program.

- A. The Old Age Pension Health Care Program provides optional benefits to clients who qualify for (State only) OAP pensions who do not qualify for Federal Financial Participation (FFP) in the Colorado Medicaid Program. These cases are coded with Supplemental Income Status Code (SISC) C.
- B. Under the Old Age Pension Health Care Program, only the following State funded benefits are provided:
 - 1. Physician and practitioner services
 - 2. Inpatient hospital
 - 3. Outpatient services
 - 4. Lab and x-ray
 - 5. Emergency transportation
 - 6. Emergency services
 - 7. Dental
 - 8. Pharmacy
 - i. Medicare Part D prescription drugs provided pursuant to the Medicare Prescription Drug, Improvement and Modernization Act of 2003 (defined at 42 U.S.C. sections 1395w-102 and 141 and 42 C.F.R. Part 423, et seq.) are not a benefit for those individuals who are eligible for both Medicare and the Old Age Pension Health Care Program. The pharmacy drug benefit under the Old Age Pension Health Care Program is subject to the requirements set forth at s Section 8.800.
 - 9. Home health services and supplies

- 10. Medicare cost sharing
 - i. If Medicare pays for a medical service that is a non-benefit under the Old Age Pension Health Care Program, the co-insurance and deductible will not be paid by the Old Age Pension Health Care Program.
- C. For the benefits listed above, the Old Age Pension Health Care Program shall only be used to provide clients with health care services determined to be medically necessary by a qualified health care provider.
- D. All other medical benefits not listed in paragraph B are excluded under the Old Age Pension Health Care Program. Inpatient care in an institution for tuberculosis or mental diseases, skilled and intermediate nursing facility services, and home and community-based services are also excluded.
- E. Eligibility shall not be retroactive and shall begin on the date of application or date eligibility is established, whichever is later.
- F. Counties shall provide information to Old Age Pension Health Care Program clients regarding the disposal of excess resources in order to qualify for the Medicaid program. Such information shall include advisements concerning the prohibition of transfer of assets without fair consideration.

8.941.2 DEFINITIONS

- A. Aid to the Needy Disabled-Colorado Supplement (AND-CS) – Program that provides a supplemental payment for individuals age zero (0) to fifty-nine (59) who are receiving Social Security Income (SSI) due to a disability or blindness, but are not receiving the full SSI benefit standard, as defined in 9 CCR 2503-5 3.510.
- B. Aid to the Needy Disabled-State Only (AND-SO) – Program that provides interim assistance to individuals age eighteen (18) through fifty-nine (59) years of age (unless diagnosed with blindness, then age zero [0] through fifty-nine [59] years of age) who are disabled or blind but have not been approved for SSI or Social Security Disability Insurance (SSDI). Individuals are required to meet the total disability requirements of the program in addition to the non-financial and financial eligibility requirements. Individuals who are partially disabled or have a short-term disability are not eligible.
- C. Federal Financial Participation (FFP) – The portion paid by the federal government to states for their share of expenditures for providing Medicaid services and for administering the Medicaid program and certain other human services programs.
- D. Medical ID Card – The card issued to members and used by providers to verify member eligibility.

- E. Old Age Pension (OAP) – Program that provides financial assistance for low-income Colorado residents who are sixty (60) years of age or older who meet all financial and non-financial eligibility requirements.
- F. Old Age Pension-C (OAP-C) – Program for individuals who are sixty (60) years of age or older who have been committed to the Colorado Mental Health Institute or to a Regional Center by order of the district or probate court.
- G. State Department or Department – The Colorado Department of Health Care Policy and Financing.
- H. Supplemental Income Status Code (SISC) – System codes used to distinguish the different types of state supplementary benefits (such as OAP) a recipient may receive. Supplemental Income Status Codes determine the FFP for benefits paid on behalf of groups covered under the Medical Assistance program.
- I. Supplemental Security Income (SSI) – A Federal income supplement program funded by general tax revenues (not Social Security taxes) that provides income to aged, blind, or disabled individuals with little or no income and resources.

8.941.3 GROUPS ASSISTED UNDER THE OLD AGE PENSION HEALTH CARE PROGRAM

Old Age Pension Health Care Program benefits are provided to persons receiving OAP who do not meet SSI eligibility criteria but do meet the State eligibility criteria for the Old Age Pension Health Care Program. These persons qualify for a SISC C..

- A. SISC C – this code is for persons eligible to receive financial assistance under OAP who do not receive an SSI payment, and do not otherwise qualify for the Colorado Medicaid Program. SISC C signifies that no FFP is available in medical assistance program expenditures.
- B. Recipients of financial assistance under AND-CS, AND-SO, or OAP-C are not eligible for assistance under the Old Age Pension Health Care Program.

8.941.4 FINANCIAL ASSISTANCE

All rules applicable to Old Age Pension financial assistance program payments (as set forth in the Department of Human Services rules at 9 CCR 2503-5) shall apply to the Old Age Pension Health Care Program.

8.941.5 CERTIFICATION OF PAYMENT FOR PROVIDERS

When submitting a claim for medical services to the Old Age Pension Health Care Program providers must submit a certification that states the following: "I will accept as payment in full, payment made under the Old Age Pension Health Care Program, and certify that no supplemental charges have been, or will be, billed to the patient, except for those non-covered items, or services, if any, which are not reimbursable under the Old Age Pension Health Care Program."

8.941.6 OUT-OF-STATE MEDICAL CARE

All requirements for out-of-state medical care as defined by Section 8.013 apply to the Old Age Pension Health Care Program for covered services with the exception that any reduction, suspension or elimination of benefits must be applied.

8.941.7 SUBMISSION OF CLAIMS

Rules governing the submission or payment of claims, provider or recipient appeals, third party liability, overpayment, fraud and abuse, and State identification numbers as defined in Section 8.100, apply to the Old Age Pension Health Care Program for covered services with the exception that any reduction, suspension or elimination of benefits provided must also be applied.

8.941.8 REIMBURSEMENT TO PROVIDERS

When reimbursement rates are modified, notifications shall be published on the Department's website and will be published in the Provider Bulletin.

8.941.9 CLIENT CO-PAYMENT

Clients are responsible for paying directly to providers a co-payment according to the regulations and fee schedule as set forth under Section 8.754.1.

Clients whose co-payments reach a limit of \$300 within a January 1 through December 31 calendar year will be exempted from further co-payments during that year. The exemption will begin on the date of payment that the \$300 limit cumulative maximum has been reached.

A client must present the Medical ID Card to the provider at the time a service is rendered in order to claim exemption from copayment for that service.

8.942 CHANGE OF SUPPLEMENTAL INCOME STATUS CODE (SISC) TO MEDICAID

8.942.1 MEDICAID QUALIFICATION

When a recipient of the Old Age Pension Health Care Program subsequently qualifies for Medicaid, their SISC must be changed to indicate Medicaid benefits. Additionally, the county must backdate the Medicaid benefits to the date the individual became eligible for Medicaid even if the recipient was eligible for the Old Age Pension Health Care Program at the time.

8.950 PRIMARY CARE FUND

8.950.1 GENERAL DESCRIPTION

8.950.1.A. In accordance with Section 21 of Article X (Tobacco Taxes for Health Related Purposes) of the State Constitution, an increase in Colorado's tax on cigarettes and tobacco products became effective January 1, 2005, and created a cash fund that was designated for health related purposes. House Bill 05-1262 divided the tobacco tax cash fund into separate funds, assigning 19% of the moneys to establish the Primary Care Fund, set forth how the funds will be allocated and designated the Department of Health Care Policy and Financing (the Department) as the administrator of the Primary Care Fund. In addition to the moneys transferred from the tobacco tax cash fund, the Primary Care Fund may also include monetary gifts, grants, and donations received pursuant to House Bill 25-1288.

8.950.1.B. The Primary Care Fund provides an allocation of Moneys to health care providers that make basic health care services available in an outpatient setting to residents of Colorado who are considered low-income and uninsured. Moneys shall be allocated based on the number of eligible patients in an amount proportionate to the total number of eligible patients served by all health care providers who qualify for Moneys from this fund.

8.950.2 DEFINITIONS

8.950.2.A. Arranges For - Demonstrating Established Referral Relationships with health care providers for any of the Comprehensive Primary Care services not directly provided by the provider.

8.950.2.B. Children's Basic Health Plan also known as Child Health Plan Plus (CHP+) - As specified in Article 8 of Title 25.5, C.R.S.

8.950.2.C. Comprehensive Primary Care - Basic, entry-level health care provided by health care practitioners or non-physician health care practitioners that is generally provided in an outpatient setting. At a minimum, this includes providing or arranging for the provision of the following services on a Year-Round Basis: primary health care; maternity care, including prenatal care; preventive, developmental, and diagnostic services for infants and children; adult preventive services, diagnostic laboratory and radiology services; emergency care for minor trauma; Pharmaceutical Services; and coordination and follow-up for hospital care. It may also include optional services based on a patient's needs such as dental, behavioral health and eyeglasses.

8.950.2.D. Cost-Effective Care - Provides or Arranges for Comprehensive Primary Care that is appropriate and at a reasonable average cost per patient Visit and/or Encounter.

8.950.2.E. Eligible Patient is a patient receiving medical services from a Qualified Provider:

1. Whose yearly family income is at or below two hundred percent (200%) of the Federal Poverty Guideline (FPG);
2. Who is not eligible for the Medical Assistance Program, the Children's Basic Health Plan, Medicare or any other governmental health care coverage such as through Social Security, the Veterans Administration, Military Dependency (TRICARE or CHAMPUS), or the United States Public Health Service; and
3. There is no Third Party Payer.

8.950.2.F Eligible Qualified Provider - A Qualified Provider who is identified by the Department to receive funding from the Primary Care Fund.

8.950.2.G. Established Referral Relationship - A formal, written agreement in the form of a letter, a memorandum of agreement or a contract between two entities which includes:

1. The Comprehensive Primary Care and/or products (e.g., pharmaceuticals, radiology) to be provided by one entity on behalf of the other entity;
2. Any applicable policies, processes or procedures;
3. The guarantee that referred Eligible Patients shall receive services on a Sliding Fee Schedule or at no charge; and
4. Signatures by representatives of both entities.

8.950.2.H. Moneys – The money the state can use for payment to providers which includes:

- Designated 19% of the tobacco tax revenue
- Monetary gifts, grants, and donations

8.950.2.I Medical Assistance Program (Medicaid) - As specified in Article 4 of Title 25.5, C.R.S.

8.950.2.J. Medically Underserved Area - A federal government designation given to a geographical area based on the ratio of medical personnel (physicians, dentists, behavioral health workers, etc.) to the population. These areas have fewer than a

generally accepted minimum number of medical personnel per thousand population resulting in insufficient health resources (personnel and/or facilities) to meet the medical needs of the resident population. Such areas are also defined by measuring the health status of the resident population; an area with an unhealthy population being considered underserved.

8.950.2.K. Medically Underserved Population - A federal government designation given to a human population that does not receive adequate medical attention or have access to health care facilities.

8.950.2.L. Outside Entity - A business or professional that is not classified as an employee of the provider or the Department and does not have a direct or indirect financial interest with the provider, but has auditing experience or experience working directly with the Medical Assistance Program or similar services or grants for Eligible Patients.

8.950.2.M. Pharmaceutical Services - Services that provide prescription drugs, or coordinate access to or Arranges for Eligible Patients to receive prescription drugs prescribed by the Qualified Provider on a Sliding Fee Schedule or at no charge.

8.950.2.N. Qualified Provider - An entity that provides Comprehensive Primary Care in Colorado and that:

1. Accepts all patients regardless of their ability to pay and uses a Sliding Fee Schedule for payments or does not charge Eligible Patients for services;
2. Serves a designated Medically Underserved Area or Medically Underserved Population as provided in section 330(b) of the federal "Public Health Service Act", 42 U.S.C. sec. 254b, or demonstrates to the Department that the entity serves a population or area that lacks adequate health care services for low-income, uninsured persons;
3. Has a demonstrated Track Record of providing Cost-Effective Care;
4. Provides or Arranges For the provision of Comprehensive Primary Care to persons of all ages. An entity in a rural area may be exempt from this requirement if they can demonstrate that there are no providers in the community to provide one or more of the Comprehensive Primary Care services;
5. Completes a screening that evaluates eligibility for the Medical Assistance Program and the Children's Basic Health Plan and refers patients potentially eligible for one of the programs to the appropriate agency (e.g., county departments of human/social services) for eligibility determination if they are not qualified to make eligibility determinations; and

6. Is a community health center, as defined in Section 330 of the federal "Public Health Services Act", 42 U.S.C. Section 254b; or at least 50% of the patients served by the provider are Eligible Patients or patients who are enrolled in the Medical Assistance Program, the Children's Basic Health Plan, or any combination thereof.

8.950.2.O. Quality Assurance Program - Formalized plan and processes designed to ensure the delivery of quality and appropriate Comprehensive Primary Care in a defined medical setting. This can be demonstrated by obtaining a certification or accreditation through the Joint Commission on Accreditation of Healthcare Organizations (JCAHO) or by the Accreditation Association for Ambulatory Health Care, Inc. (AAAHHC). If such certification or accreditation is not available, then at a minimum, the Quality Assurance Program shall be comprised of elements that meet or exceed the following components:

1. Establishment of credentialing/re-credentialing requirements for medical personnel;
2. Surveying and monitoring of patient satisfaction;
3. Establishment of a grievance process for patients, including documentation of grievances and resolutions;
4. Development of clinic operating policies and scheduled performance monitoring;
5. Review of medical records to check for compliance with established policies and to monitor quality of care;
6. Assessment of state and federal regulations to ensure compliance;
7. Establishment of patient safety procedures; and
8. Establishment of infection control practices.

8.950.2.P. Sliding Fee Schedule - A tiered co-payment system that determines the level of patient's financial participation and guarantees that the patient financial participation is below usual and customary charges. Factors considered in establishing the tiered co-payment system shall only be financial status and the number of members in the patient's family unit.

8.950.2.Q. Third Party Payments or Third Party Payer - Any individual, entity or program with a legal obligation to pay for some or all health-related services rendered to a patient. Examples include the Medical Assistance Program; the Children's Basic Health Plan; Medicare; commercial, individual or employment-related health insurance; court-ordered health insurance (such as that required by non-custodial parents); workers' compensation; automobile insurance; and long-term care insurance.

8.950.2.R. Track Record - Evidence of providing Comprehensive Primary Care covering at least a consecutive 52-week period prior to the submission of the application.

8.950.2.S. Unduplicated User/Patient Count - The sum of patients who have had at least one Visit/Encounter and received at least one of the services under the Comprehensive Primary Care definition during the applicable calendar year, but does not include the same patient more than once. The sum shall be calculated on a specific point-in-time occurring between the end of the applicable calendar year and prior to the submission of the application. Each patient shall be counted once under only one payment source designation (Third Party Payer or Eligible Patient). The patient's payment source designation shall be the payment source designation listed for the patient at the specific point-in-time in which the calculation is made. The sum shall not include:

1. Counting a patient more than once if the same patient returns for additional services (e.g., medical or dental) and/or products (e.g., pharmaceuticals) during the applicable calendar year;
2. Counting a patient more than once if the payment source designation changed during the applicable calendar year;
3. Persons who have only received services through an outreach event, community education program, nurse hotline, or other types of community-based events or programs and were not documented on an individual basis;
4. Persons who have only received services from large-scale efforts such as mass immunization programs, screening programs, and health fairs; or
5. Persons whose only contact with the provider is to receive Special Supplemental Nutrition Program for Women, Infants, and Children (WIC Program) counseling and vouchers are not users and the contact does not generate an encounter.

8.950.2.T. Visit/Encounter - An appointment with medical personnel (physicians, physician assistants, dentists, behavioral health workers, etc.) in which the patient received health related services and/or products (e.g., pharmaceuticals or radiology) and the appointment is customarily billable to a Third Party Payer.

8.950.2.U. Year-Round Basis - Comprehensive Primary Care provided in a consecutive 52-week period directly by the provider and/or through an established referral relationship with other providers. If an organization is closed for four consecutive weeks or longer in a calendar year on a regularly scheduled basis, it is not considered to directly provide services on a year-round basis.

8.950.3 PROVIDER ELIGIBILITY

8.950.3.A. Providers who provide Comprehensive Primary Care to Eligible Patients and who meet all of the requirements established for the Primary Care Fund as of the date the application form is submitted to the Department shall receive moneys appropriated to the Primary Care Fund. Specifically, the provider shall:

1. Meet all of the requirements of a Qualified Provider as specified in Section 8.950.2.N;
2. Have a Quality Assurance Program in place as specified in Section 8.950.2.O; and
3. Submit a completed application form according to stated guidelines as specified under Section 8.950.4.

8.950.4 APPLICATION

8.950.4.A. The application form shall be available to providers annually and posted for public access on the Department's website at least 30 calendar days prior to the response due date.

8.950.4.B. At a minimum, the application form shall require responses that:

1. Demonstrate how the provider meets the criteria of a Qualified Provider as defined in Section 8.950.2.N;
2. Provide an Unduplicated User/Patient Count covering the applicable calendar year which, at a minimum, shall include the number of patients eligible for the Medical Assistance Program and the Children's Basic Health Plan and the number of patients considered to be Eligible Patients;
3. Provide certification that the Unduplicated User/Patient Count identified in Section 8.950.4.B.2 has been verified by an Outside Entity; and
4. Provide documentation that the provider has a Quality Assurance Program as defined in Section 8.950.2.O.

8.950.4.C. Providers shall complete and provide a response annually. The response shall be made in compliance with all specifications in the application form, including format, data and documentation. Responses to the application form shall be submitted directly to the Department by the required response deadline.

8.950.4.D. All providers who submit a response to the application form shall be notified within 45 days of the response deadline if the provider met or did not meet the requirements to become an Eligible Qualified Provider.

8.950.5 DISBURSEMENT

8.950.5.A. Eligible Qualified Providers are determined on a state fiscal year basis and shall receive only those Moneys appropriated to the Primary Care Fund for that same state fiscal year, subject to the tax amount actually collected for that state fiscal year and all monetary gifts, grants and donations received during the state fiscal year.

8.950.5.B. Payments shall be based on the number of Eligible Patients in each Eligible Qualified Provider's Unduplicated User/Patient Count in an amount proportionate to the total number of Eligible Patients from all Eligible Qualified Providers' Unduplicated User/Patient Counts.

8.950.5.C. The schedule for the disbursement of Moneys to all Eligible Qualified Providers shall be dependent on actual tax collections allocated to the Primary Care Fund such that:

1. Tax collections for sales and monetary gifts, grants and donations received in July, August, and September shall be distributed to Eligible Qualified Providers prior to the end of October.
2. Tax collections for sales and monetary gifts, grants and donations received in October, November, and December shall be distributed to Eligible Qualified Providers prior to the end of January.
3. Tax collections for sales and monetary gifts, grants and donations received in January, February, and March shall be distributed to Eligible Qualified Providers prior to the end of April.
4. Tax collections for sales and monetary gifts, grants and donations received in April, May, and June shall be distributed to Eligible Qualified Providers prior to the end of July.
5. For State Fiscal Year 2005-06 only, tax collections for sales in January 2005 through December 2005, shall be distributed to Eligible Qualified Providers prior to the end of February 2006.

8.960 COLORADO DENTAL HEALTH CARE PROGRAM FOR LOW-INCOME SENIORS

8.960.A Definitions

1. Arrange For or Arranging For means demonstrating established relations with Qualified Providers for any of the Covered Dental Care Services not directly provided by the applicant.
2. Covered Dental Care Services include Diagnostic Imaging, Emergency Services, Endodontic Services, Evaluation, Oral and Maxillofacial Surgery, Palliative

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- Treatment, Periodontal Treatment, Preventive Services, Prophylaxis, Removable Prosthesis, and Restorative Services as listed by alphanumeric procedure code in Schedule A.
3. C.R.S. means the Colorado Revised Statutes.
 4. Dental Health Professional Shortage Area or Dental HPSA means a geographic area, population group, or facility so designated by the Health Resources and Services Administration of the U.S. Department of Health and Human Services.
 5. Dental Prosthesis means any device or appliance replacing one or more missing teeth and associated structures if required.
 6. Department means the Colorado Department of Health Care Policy and Financing established pursuant to title 25.5, C.R.S.
 7. Diagnostic Imaging means a visual display of structural or functional patterns for the purpose of diagnostic evaluation.
 8. Economically Disadvantaged means a person whose Income is at or below 250% of the most recently published federal poverty level for a household of that size.
 9. Eligible Senior or patient means an adult who is 60 years of age or older, who is Economically Disadvantaged, who is not eligible for dental services under Medicaid or the Old Age Pension Health and Medical Care Program, and who does not have private dental insurance. An Eligible Senior or patient is not ineligible solely because he/she is receiving dental benefits under Medicare or Medicare Advantage Plans.
 10. Emergency Services means the need for immediate intervention by a Qualified Provider to stabilize an oral cavity condition.
 11. Endodontic Services means services which are concerned with the morphology, physiology and pathology of the human dental pulp and periradicular tissues, including pulpectomy.
 12. Evaluation means an assessment that may include gathering of information through interview, observation, examination, and use of specific tests that allows a dentist to diagnose existing conditions.
 13. Federally Qualified Health Center means a federally funded nonprofit health center or clinic that serves medically underserved areas and populations as defined in 42 U.S.C. section 1395x (aa)(4).
 14. Income means any cash, payments, wages, in-kind receipt, inheritance, gift, prize, rents, dividends, or interest that are received by an individual or family. Income may be self-declared. Resources are not included in Income.
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15. Max Allowable Fee means the total reimbursement listed by procedure for Covered Dental Care Services under the Colorado Dental Health Care Program for Low-Income Seniors in Schedule A. The Max Allowable Fee is the sum of the Program Payment and the Max Patient Co-Pay.
16. Max Patient Co-Pay means the maximum amount that a Qualified Provider may collect from an Eligible Senior listed by procedure in Schedule A for Covered Dental Services under the Colorado Dental Health Care Program for Low-Income Seniors.
17. Medicaid means the Colorado medical assistance program as defined in article 4 of title 25.5, C.R.S.
18. Medicare means the federal health insurance program for people who are 65 or older; certain younger people with disabilities; or people with End-Stage Renal Disease.
19. Medicare Advantage Plans mean the plans offered by Medicare-approved private companies that must follow rules set by Medicare and may provide benefits for services Medicare does not, such as vision, hearing, and dental care.
20. Old Age Pension Health and Medical Care Program means the program described at Section 8.940 et. seq. and as defined in sections 25.5-2-101 and 26-2-111(2), C.R.S.
21. Oral and Maxillofacial Surgery means the diagnosis, surgical and adjunctive treatment of diseases, injuries and defects involving both the functional and esthetic aspects of the hard and soft tissues of the oral and maxillofacial region.
22. Palliative Treatment for dental pain means emergency treatment to relieve the patient of pain; it is not a mechanism for addressing chronic pain.
23. Periodontal Treatment means the therapeutic plan intended to stop or slow periodontal disease progression.
24. Preventive Services means services concerned with promoting good oral health and function by preventing or reducing the onset and/or development of oral diseases or deformities and the occurrence of oro-facial injuries.
25. Program Payment means the maximum amount by procedure listed in Schedule A for Covered Dental Care Services for which a Qualified Grantee may invoice the Department under the Colorado Dental Health Care Program for Low-Income Seniors. Program Payment must not be less than the reimbursement schedule for fee-for-service dental fees under the medical assistance program established in Articles 4, 5, and 6 of 10 CCR 2505-10.
26. Prophylaxis means the removal of dental plaque and calculus from teeth, in order to prevent dental caries, gingivitis and periodontitis.

- 27. Qualified Grantee means an entity that can demonstrate that it can provide or Arrange For the provision of Covered Dental Care Services and may include but is not limited to:
 - a. An Area Agency on Aging, as defined in section 26-11-201, C.R.S.;
 - b. A community-based organization or foundation;
 - c. A Federally Qualified Health Center, safety-net clinic, or health district;
 - d. A local public health agency; or
 - e. A private dental practice.
- 28. Qualified Provider means a licensed dentist or dental hygienist in good standing in Colorado or a person who employs a licensed dentist or dental hygienist in good standing in Colorado and who is willing to accept reimbursement for Covered Dental Services. A Qualified Provider may also be a Qualified Grantee if the person meets the qualifications of a Qualified Grantee.
- 29. Removable Prosthesis means complete or partial Dental Prosthesis, which after an initial fitting by a dentist, can be removed and reinserted by the eligible senior.
- 30. Restorative Services means services rendered for the purpose of rehabilitation of dentition to functional or aesthetic needs of the patient.
- 31. Senior Dental Advisory Committee means the advisory committee established pursuant to section 25.5-3-406, C.R.S..

8.960.B Legal Basis

8.960.B.1 The Colorado Dental Health Care Program for Low-Income Seniors is authorized by state law at part 4 of article 3 of title 25.5, C.R.S..

8.960.C Request of Grant Proposals and Grant Award Procedures

8.960.C.1. Request for Grant Proposals

8.960.C.1.a Grant awards shall be made through an application process. The request for grant proposals form shall be issued by the Department and posted for public access on the Department's website at <https://www.colorado.gov/hcpf/research-data-and-grants> at least 30 days prior to the due date.

8.960.C.2 Evaluation of Grant Proposals

8.960.C.2.a Proposals submitted for the Colorado Dental Health Care Program for Low-Income Seniors will be evaluated by a review panel in accordance with the following criteria developed under the advice of the Senior Dental Advisory Committee.

- 1) The review panel will be comprised of individuals who are deemed qualified by reason of training and/or experience and who have no personal or financial interest in the selection of any particular applicant.
- 2) The sole objective of the review panel is to recommend to the Department's executive director those proposals which most accurately and effectively meet the goals of the program within the available funding.
- 3) Preference will be given to grant proposals that clearly demonstrate the applicant's ability to:
 - a) Outreach to and identify Eligible Seniors;
 - b) Collaborate with community-based organizations; and
 - c) Serve a greater number of Eligible Seniors or serve Eligible Seniors who reside in a geographic area designated as a Dental HPSA.
- 4) The review panel shall consider the distribution of funds across the state in recommending grant proposals for awards. The distribution of funds should be based on the estimated percentage of Eligible Seniors in the state by Area Agency on Aging region as provided by the Department.

8.960.C.3 Grant Awards

8.960.C.3.a The Department's executive director, or his or her designee, shall make the final grant awards to selected Qualified Grantees for the Colorado Dental Health Care Program for Low-Income Seniors.

8.960.C.4 Qualified Grantee Responsibilities

8.960.C.4.a A Qualified Grantee that is awarded a grant under the Colorado Dental Health Care Program for Low-Income Seniors is required to:

- 1) Identify and outreach to Eligible Seniors and Qualified Providers;
- 2) Demonstrate collaboration with community-based organizations;
- 3) Ensure that Eligible Seniors receive Covered Dental Care Services efficiently without duplication of services;
- 4) Maintain records of Eligible Seniors serviced, Covered Dental Care Services provided, and moneys spent for a minimum of six (6) years;
- 5) For Eligible Seniors with dental coverage through a Medicare Advantage Plan, bill the Medicare Advantage Plan for dental procedures covered by the Medicare Advantage Plan prior to seeking payment from the

Department. The Colorado Dental Health Care Program is secondary to the Medicare Advantage Plan dental coverage;

- 6) Distribute grant funds to Qualified Providers in its service area or directly provide Covered Dental Care Services to Eligible Seniors;
- 7) Expend no more than seven (7) percent of the amount of its grant award for administrative purposes; and
- 8) Submit an annual report as specified under section 8.960.3.F.

8.960.C.5 Invoicing

8.960.C.5.a A Qualified Grantee that is awarded a grant under the Colorado Dental Health Care Program for Low-Income Seniors shall submit invoices on a form and schedule specified by the Department. Covered Dental Care Services shall be provided before a Qualified Grantee may submit an invoice to the Department.

- 1) Invoices shall include the number of Eligible Seniors served, the alphanumeric code and procedure description as listed in Schedule A, and any other information required by the Department.
- 2) The Department will pay no more than the established Program Payment per procedure rendered, as listed in Schedule A.
- 3) Eligible Seniors shall not be charged more than the Max Patient Co-Pay as listed in Schedule A.
- 4) Qualified Grantees shall not bill the Department for any procedures covered by Medicare Advantage Plans that have been billed and paid by the Medicare Advantage Plans;
- 5) Qualified Grantees shall indicate on the invoice if the Eligible Senior has dental coverage through a Medicare Advantage Plan and any claim to the Medicare Advantage Plan was adjudicated prior to billing the Department;
- 6) Qualified Grantees may invoice for no more than seven (7) percent of the Program Payment for administrative costs.

8.960.C.6 Annual Report

8.960.C.6.a On or before September 1, 2016, and each September 1 thereafter, each Qualified Grantee receiving funds from the Colorado Dental Health Care Program for Low-Income Seniors shall submit a report to the Department following the state fiscal year contract period.

8.960.C.6.b The annual report shall be completed in a format specified by the Department and shall include:

- 1) The number of Eligible Seniors served;
- 2) The types of Covered Dental Care Services provided;
- 3) An itemization of administrative expenditures;
- 4) The procedures and amounts billed to Medicare Advantage Plans for Eligible Seniors; and
- 5) Any other information deemed relevant by the Department.

**10 CCR 2505-10 § 8.960 SCHEDULE A: COLORADO DENTAL HEALTH CARE
PROGRAM FOR LOW-INCOME SENIORS COVERED SERVICES AND PROCEDURE
CODES**

Capitalized terms within this schedule shall have the meaning specified in the Definitions section.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Periodic oral evaluation - established patient	D0120	\$32.41	\$32.41	\$0.00		Evaluation performed on a patient of record to determine any changes in the patient's dental and medical health status since a previous comprehensive or periodic evaluation. This includes an oral cancer evaluation, periodontal screening where indicated, and may require interpretation of information acquired through additional diagnostic procedures. The findings are discussed with the patient. Report additional diagnostic procedures separately. Frequency: Two of D0120, D0150, D0180 per 12 months per patient.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Limited oral evaluation - problem focused	D0140	\$54.90	\$44.90	\$10.00		An evaluation limited to a specific oral health problem or complaint. This may require interpretation of information acquired through additional diagnostic procedures. Report additional diagnostic procedures separately. Definitive procedures may be required on the same date as the evaluation. Typically, patients receiving this type of evaluation present with a specific problem and/or dental emergencies, trauma, acute infections, etc. Frequency: Two of D0140 per 12 months per grantee per patient. Not reimbursable on the same date as D0120, D0150, or D0180. Dental hygienists may only provide for an established patient of record.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Comprehensive oral evaluation - new or established patient	D0150	\$51.57	\$51.57	\$0.00		Evaluation used by general dentist and/or a specialist when evaluating a patient comprehensively. Applicable to new patients; established patients with significant health changes or other unusual circumstances by report; or established patients who have been absent from active treatment for three or more years. It is a thorough evaluation and recording of the extraoral and intraoral hard and soft tissues. It may require interpretation of information acquired through additional diagnostic procedures. Additional diagnostic procedures should be reported separately. This includes an evaluation for oral cancer, the evaluation and recording of the patient's dental and medical history and general health assessment. It may include the evaluation and recording of dental caries, missing or unerupted teeth, restorations, existing prostheses, occlusal relationships, periodontal conditions (including periodontal screening and/or charting), hard and soft tissue anomalies, etc. Frequency: One of D0150 per 36 months per grantee per patient. Two of D0120, D0150, D0180 per 12 months per grantee per patient.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Comprehensive periodontal evaluation - new or established patient	D0180	\$45.04	\$45.04	\$0.00		This procedure is indicated for patients showing signs & symptoms of periodontal disease & patients with risk factors such as smoking or diabetes. It includes A comprehensive evaluation of periodontal conditions, including full mouth probing and periodontal charting. Indicated for patients exhibiting signs or symptoms of periodontal disease, systemic medical conditions, or patients with social risk factors. It includes an evaluation for oral cancer, an evaluation of the patient's medical history, a general assessment, and includes an evaluation of current dental conditions. Frequency: One of D0180 per 36 months per patient. Two of D0120, D0150, D0180 per 12 months per patient.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Intraoral - comprehensive series of radiographic images	D0210	\$88.31	\$88.31	\$0.00		Radiographic survey of whole mouth, intended to display the crowns & roots of all teeth, periapical areas, interproximal areas and alveolar bone including edentulous areas. Panoramic radiographic image D0330 & bitewing radiographic images D0270-D0277 taken on the same date of service shall not be billed as a D0210. Minimum of 12-20 films is required. Payment for additional periapical radiographs within 60 days of a full mouth series D0277 or a panoramic film D0330 is not covered unless there is evidence of trauma. Frequency: One of D0210, D0277, D0330 per 60 months per patient. Any combination of x-rays taken on the same date of service that equals or exceeds the max allowable fee for D0210 must be billed and reimbursed as D0210.
Intraoral - periapical first radiographic image	D0220	\$13.21	\$13.21	\$0.00		Six of D0220 per 12 months per patient. Report additional radiographs as D0230. Working and final endodontic treatment films are not covered. Not covered if billed with D3310, D3320, D3330. Any combination of D0220 through D0277 taken on the same date of service that exceeds the max allowed fee for D0210 is reimbursed at the same fee as D0210. Not allowed on the same day as D0210.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Intraoral - periapical each additional radiographic image	D0230	\$13.21	\$13.21	\$0.00		D0230 must be utilized for additional films taken beyond D0220. Working and final endodontic treatment films are included in the endo codes. Not covered if billed with D3310, D3320, or D3330. Not allowed on the same day as D0210. Any combination of D0220 through D0277 taken on the same date of service that exceeds the max allowed fee for D0210 is reimbursed at the same fee as D0210.
Bitewing - single radiographic image	D0270	\$13.79	\$13.79	\$0.00		Frequency: One of D0270, D0272, D0273, D0274 per 12 months per patient. Any combination of D0220 through D0277 taken on the same date of service that exceeds the max allowed fee for D0210 is reimbursed at the same fee as D0210.
Bitewings - two radiographic images	D0272	\$22.20	\$22.20	\$0.00		Frequency: One of D0270, D0272, D0273, D0274 per 12 months per patient. Any combination of D0220 through D0277 taken on the same date of service that exceeds the max allowed fee for D0210 is reimbursed at the same fee as D0210.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Bitewings - three radiographic images	D0273	\$26.17	\$26.17	\$0.00		Frequency: One of D0270, D0272, D0273, D0274 per 12 months per patient. Any combination of D0220 through D0277 taken on the same date of service that exceeds the max allowed fee for D0210 is reimbursed at the same fee as D0210.
Bitewings - four radiographic images	D0274	\$31.20	\$31.20	\$0.00		Frequency: One of D0270, D0272, D0273, D0274 per 12 months per patient. Any combination of D0220 through D0277 taken on the same date of service that exceeds the max allowed fee for D0210 is reimbursed at the same fee as D0210.
Vertical bitewings – seven to eight radiographic images	D0277	\$46.25	\$46.25	\$0.00		Frequency: One of D0210, D0277, D0330 per 60 months per patient. Counts as a full mouth series. Counts as an intraoral complete series. Counts as an intraoral complete series. Any combination of D0220 through D0277 taken on the same date of service that exceeds the max allowed fee for D0210 is reimbursed at the same fee as D0210.
Panoramic radiographic image	D0330	\$55.21	\$55.21	\$0.00		Frequency: One of D0210, D0277, D0330 per 60 months per grantee per patient. Counts as a full mouth series.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Prophylaxis - adult	D1110	\$82.39	\$82.39	\$0.00		Removal of plaque, calculus and stains from the tooth structures and implants in the permanent and transitional dentition. It is intended to control local irritational factors. Frequency: • Two of D1110, D4346, D4910 per 12 months per patient. • • • May be alternated with D4910 for maintenance of periodontally-involved individuals. • D1110 cannot be billed on the same day as D4341 – D4910. • Only allowed for cases with a history of surgical or non-surgical periodontal treatment, excluding D4355.
Topical application of fluoride varnish	D1206	\$35.46	\$35.46	\$0.00		Topical fluoride application is to be used in conjunction with prophylaxis or preventive appointment. Should be applied to whole mouth. Frequency: up to four times per 12 calendar months. Cannot be used with D1208.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Topical application of fluoride - excluding varnish	D1208	\$12.25	\$12.25	\$0.00		Any fluoride application, including swishing, trays or paint on variety, to be used in conjunction with prophylaxis or preventive appointment. Frequency: one time per 12 calendar months. Cannot be used with D1206. D1206 varnish should be utilized in lieu of D1208 whenever possible.
Application of caries arresting medicament – per tooth	D1354	\$46.08	\$46.08	\$0.00	Teeth 1-32	Conservative treatment of an active, non-symptomatic carious lesion by topical application of a caries arresting or inhibiting medicament and without mechanical removal of sound tooth structure. Frequency: Two of D1354 per 12 months per patient per tooth for permanent teeth. Not to exceed 4 times per tooth in a lifetime. Cannot be billed on the same day as D1355 or D3110 or D3120 or any D2000 series code (D2140–D2954). Must Report tooth number.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Caries preventive medicament application – per tooth	D1355	\$5.83	\$5.83	\$0.00	Teeth 1-32	For primary prevention or remineralization. Medicaments applied do not include topical fluorides. Medicaments that may be applied during the delivery of D1355 procedure include Silver Diamine Fluoride (SDF), Silver Nitrate (SN), thymol-CHX varnish, and topical povidone iodine (PVP-I). Cannot be billed on the same day as: D1206, D1208, D1354, D0140, D9110, or any restoration codes on the same day or within 12 months of D2140 thru D2954. Maximum of four D1355 per tooth per lifetime. Must report tooth number.
Amalgam - one surface, primary or permanent	D2140	\$120.02	\$110.02	\$10.00	Teeth 1-32	Frequency: One of D2140 – D2394 per 36 months per patient per tooth per surface.
Amalgam - two surfaces, primary or permanent	D2150	\$150.59	\$140.59	\$10.00	Teeth 1-32	Frequency: One of D2140 – D2394 per 36 months per patient per tooth per surface.
Amalgam - three surfaces, primary or permanent	D2160	\$182.40	\$172.40	\$10.00	Teeth 1-32	Frequency: One of D2140 – D2394 per 36 months per patient per tooth per surface.
Amalgam - four or more surfaces, primary or permanent	D2161	\$218.93	\$208.93	\$10.00	Teeth 1-32	Frequency: One of D2140 – D2394 per 36 months per patient per tooth per surface.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Resin-based composite - one surface, anterior	D2330	\$116.82	\$106.82	\$10.00	Teeth 6 - 11, 22 - 27	Frequency: One of D2140 – D2394 per 36 months per patient per tooth per surface. See Explanation of Restorations.
Resin-based composite - two surfaces, anterior	D2331	\$142.18	\$132.18	\$10.00	Teeth 6 - 11, 22 - 27	Frequency: One of D2140 – D2394 per 36 months per patient per tooth per surface. See Explanation of Restorations.
Resin-based composite - three surfaces, anterior	D2332	\$172.09	\$162.09	\$10.00	Teeth 6 - 11, 22 – 27	Frequency: One of D2140 – D2394 per 36 months per patient per tooth per surface. See Explanation of Restorations.
Resin-based composite - four or more surfaces (anterior)	D2335	\$205.55	\$195.55	\$10.00	Teeth 6 - 11, 22 - 27	. Frequency: One of D2140 – D2394 per 36 months per patient per tooth per surface. See Explanation of Restorations.
Resin-based composite - one surface, posterior	D2391	\$120.02	\$110.02	\$10.00	Teeth 1 - 5, 12 - 21, 28 - 32	Frequency: One of D2140 – D2394 per 36 months per patient per tooth per surface. See Explanation of Restorations.
Resin-based composite - two surfaces, posterior	D2392	\$150.59	\$140.59	\$10.00	Teeth 1 - 5, 12 - 21, 28 - 32	Frequency: One of D2140 – D2394 per 36 months per patient per tooth per surface. See Explanation of Restorations.
Resin-based composite - three surfaces, posterior	D2393	\$182.40	\$172.40	\$10.00	Teeth 1 - 5, 12 - 21, 28 - 32	Frequency: One of D2140 – D2394 per 36 months per patient per tooth per surface. See Explanation of Restorations.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Resin-based composite - four or more surfaces, posterior	D2394	\$218.93	\$208.93	\$10.00	Teeth 1 - 5, 12 - 21, 28 - 32	Frequency: One of D2140 – D2394 per 36 months per patient per tooth per surface. See Explanation of Restorations.
Crown - porcelain/ceramic	D2740	\$767.54	\$717.54	\$50.00	Teeth 2 - 15, 18 - 31	Frequency: One of D2740 – D2794 per 84 months per patient per tooth. Second molars are only covered if it meets criteria and is necessary to support a partial denture or to maintain eight posterior teeth in occlusion.
Crown - porcelain fused to high noble metal	D2750	\$760.70	\$710.70	\$50.00	Teeth 2 - 15, 18 - 31	Frequency: One of D2740 – D2794 per 84 months per patient per tooth. Second molars are only covered if it meets criteria and is necessary to support a partial denture or to maintain eight posterior teeth in occlusion.
Crown - porcelain fused to predominantl y base metal	D2751	\$698.14	\$648.14	\$50.00	Teeth 2 - 15, 18 - 31	Frequency: One of D2740 – D2794 per 84 months per patient per tooth. Second molars are only covered if it meets criteria and is necessary to support a partial denture or to maintain eight posterior teeth in occlusion.
Crown - porcelain fused to noble metal	D2752	\$724.56	\$674.56	\$50.00	Teeth 2 - 15, 18 - 31	Frequency: One of D2740 – D2794 per 84 months per patient per tooth. Second molars are only covered if it meets criteria and is necessary to support a partial denture or to maintain eight posterior teeth in occlusion.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Crown - 3/4 cast predominantl y base metal	D2781	\$542.27	\$492.27	\$50.00	Teeth 2 - 15, 18 - 31	Frequency: One of D2740 – D2794 per 84 months per patient per tooth. Second molars are only covered if it meets criteria and is necessary to support a partial denture or to maintain eight posterior teeth in occlusion.
Crown - 3/4 cast noble metal	D2782	\$542.27	\$492.27	\$50.00	Teeth 2 - 15, 18 - 31	Frequency: One of D2740 – D2794 per 84 months per patient per tooth. Second molars are only covered if it meets criteria and is necessary to support a partial denture or to maintain eight posterior teeth in occlusion.
Crown - 3/4 porcelain/cer amic	D2783	\$542.27	\$492.27	\$50.00	Teeth 1 - 32	This procedure does not include facial veneers. Frequency: One of D2740 – D2794 per 84 months per patient per tooth. Second molars are only covered if it meets criteria and is necessary to support a partial denture or to maintain eight posterior teeth in occlusion.
Crown - full cast high noble metal	D2790	\$783.98	\$733.98	\$50.00	Teeth 2 - 15, 18 - 31	Frequency: One of D2740 – D2794 per 84 months per patient per tooth. Second molars are only covered if it meets criteria and is necessary to support a partial denture or to maintain eight posterior teeth in occlusion.
Crown - full cast predominantl y base metal	D2791	\$542.27	\$492.27	\$50.00	Teeth 2 - 15, 18 - 31	Frequency: One of D2740 – D2794 per 84 months per patient per tooth. Second molars are only covered if it meets criteria and is necessary to support a partial denture or to maintain eight posterior teeth in occlusion.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Crown - full cast noble metal	D2792	\$542.27	\$492.27	\$50.00	Teeth 2 - 15, 18 - 31	Frequency: One of D2740 – D2794 per 84 months per patient per tooth. Second molars are only covered if it meets criteria and is necessary to support a partial denture or to maintain eight posterior teeth in occlusion.
Crown titanium and titanium alloys	D2794	\$757.16	\$707.16	\$50.00	Teeth 2 - 15, 18 - 31	Frequency: One of D2740 – D2794 per 84 months per patient per tooth. Second molars are only covered if it meets criteria and is necessary to support a partial denture or to maintain eight posterior teeth in occlusion.
Re-cement or re-bond inlay, onlay, veneer or partial coverage restoration	D2910	\$62.21	\$52.21	\$10.00	Teeth 1 - 32	Not allowed within 6 months of placement.
Re-cement or re-bond crown	D2920	\$63.44	\$53.44	\$10.00	Teeth 1 - 32	Not allowed within 6 months of placement.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Placement of Interim Direct Restoration	D2940	\$65.21	\$55.21	\$10.00	Teeth 1 - 32	Direct placement of a restorative material to protect tooth and/or tissue form. This procedure may be used to relieve pain, promote healing, manage caries, create a seal for endodontic isolation, or prevent further deterioration until definitive treatment can be rendered. Not to be used for endodontic access closure, or as a base or liner under restoration. One of D2940 per lifetime per tooth. RDH's will receive reimbursement when used for telehealth dentistry in partnership with treating dentist.
Core buildup, including any pins when required	D2950	\$160.082 25.00	\$135.08	\$25.00	Teeth 2 - 15, 18 - 31	. Refers to building up of coronal structure when there is insufficient retention for a separate extracoronary restorative procedure. A core buildup is not a filler to eliminate any undercut, box form, or concave irregularity in a preparation. Frequency: One of D2950, D2952, D2954 per 84 months per patient per tooth. Refers to building up of anatomical crown when restorative crown will be placed. Not payable on the same tooth and same day as D2951.
Pin retention - per tooth, in addition to restoration	D2951	\$43.02	\$33.02	\$10.00	Teeth 2 - 15, 18 - 31	Pins placed to aid in retention of restoration. Can only be used in combination with a multi-surface amalgam.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Post and core in addition to crown, indirectly fabricated	D2952	\$232.12	\$207.12	\$25.00	Teeth 2 - 15, 18 - 31	Post and core are custom fabricated as a single unit. Frequency: One of D2950, D2952, D2954 per 84 months per patient per tooth. Refers to building up of anatomical crown when restorative crown will be placed. Not payable on the same tooth and same day as D2951.
Prefabricated post and core in addition to crown	D2954	\$189.50	\$164.50	\$25.00	Teeth 2 - 15, 18 - 31	Core is built around a prefabricated post. This procedure includes the core material. Frequency: One of D2950, D2952, D2954 per 84 months per patient per tooth. Refers to building up of anatomical crown when restorative crown will be placed. Not payable on the same tooth and same day as D2951.
Application of Hydroxyapatite Regeneration Medicament - per tooth	D2991	\$66.30	\$56.30	\$10.00	Teeth 1 - 32	Preparation of tooth surfaces and topical application of a scaffold to guide hydroxyapatite regeneration. One of D2991 per lifetime per patient per tooth. Cannot be billed on the same day/same tooth as any other D2000's codes or D1354.
Endodontic therapy, anterior tooth (excluding final restoration)	D3310	\$725.80	\$675.80	\$50.00	Teeth 6 - 11, 22 - 27	Frequency: One D3310 per lifetime per patient per tooth. Teeth covered: 6-11 and 22-27.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Endodontic therapy, premolar tooth (excluding final restoration)	D3320	\$825.46	\$775.46	\$50.00	Teeth 4, 5, 12, 13, 20, 21, 28, 29	Frequency: One D3320 per lifetime per patient per tooth. Teeth covered: 4, 5, 12, 13, 20, 21, 28, and 29.
Endodontic therapy, molar tooth (excluding final restoration)	D3330	\$987.37	\$937.37	\$50.00	Teeth 2, 3, 14, 15, 18, 19, 30, 31	Frequency: One D3330 per lifetime per patient per tooth. Second molars are only covered if it meets criteria and is necessary to support a partial denture or to maintain eight posterior teeth in occlusion. Teeth covered: 2, 3, 14, 15, 18, 19, 30, and 31.
Retreatment of Previous Root Canal Therapy- Anterior	D3346	\$820.31	\$770.31	\$50.00	Teeth 6 - 11, 22 - 27	Includes all appointments necessary to complete treatment; also includes intra-operative radiographs. Does not include diagnostic evaluation and necessary radiographs/diagnostic images. One of D3346 per lifetime per patient per tooth. Only reimbursable if original treatment not paid by Senior Dental Program.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Retreatment of Previous Root Canal Therapy-Premolar	D3347	\$932.28	\$882.28	\$50.00	Teeth 4, 5, 12, 13, 20, 21, 28, 29	Includes all appointments necessary to complete treatment; also includes intra-operative radiographs. Does not include diagnostic evaluation and necessary radiographs/diagnostic images. One of D3347 per lifetime per patient per tooth. Only reimbursable if original treatment not paid by Senior Dental Program
Retreatment of Previous Root Canal Therapy-Molar	D3348	\$1,102.92	\$1,052.92	\$50.00	Teeth 2, 3, 14, 15, 18, 19, 30, 31	Includes all appointments necessary to complete treatment; also includes intra-operative radiographs. Does not include diagnostic evaluation and necessary radiographs/diagnostic images. One of D3348 per lifetime per patient per tooth. Only reimbursable if original treatment not paid by Senior Dental Program. Second molars are only covered if it meets criteria and is necessary to support a partial denture or to maintain eight posterior teeth in occlusion.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Periodontal scaling & root planing - four or more teeth per quadrant	D4341	\$235.20	\$225.20	\$10.00	Per Quadrant LL, LR, UL, or UR	This procedure involves instrumentation of the crown and root surfaces of the teeth to remove plaque and calculus from these surfaces. It is indicated for patients with periodontal disease and is therapeutic, not prophylactic, in nature. Root planing is the definitive procedure designed for the removal of cementum and dentin that is rough, and/or permeated by calculus or contaminated with toxins or microorganisms. Some soft tissue removal occurs. This procedure may be used as a definitive treatment in some stages of periodontal disease and/or as part of pre-surgical procedures in others. Frequency: • One of D4341, D4342 per 36 months per patient per quadrant. A minimum of four affected teeth in the quadrant. • Maximum of two quadrants per date of service in a non-hospital setting. • Cannot be charged on same date as D1110. • Any follow-up and re-evaluation are included in the initial reimbursement.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Periodontal scaling & root planing - one to three teeth per quadrant	D4342	\$160.28	\$160.28	\$0.00	Per Quadrant LL, LR, UL, or UR	This procedure involves instrumentation of the crown and root surfaces of the teeth to remove plaque and calculus from these surfaces. It is indicated for patients with periodontal disease and is therapeutic, not prophylactic, in nature. Root planing is the definitive procedure designed for the removal of cementum and dentin that is rough, and/or permeated by calculus or contaminated with toxins or microorganisms. Some soft tissue removal occurs. This procedure may be used as a definitive treatment in some stages of periodontal disease and/or as part of pre-surgical procedures in others. Current periodontal charting must be present in patient chart documenting active periodontal disease. Frequency: • One of D4341, D4342 per 36 months per patient per quadrant. A maximum of three teeth in the affected quadrant. • Maximum of two quadrants per date of service in a non-hospital setting.. Documentation of other treatment provided at same time will be requested. • Cannot be charged on same date as D1110. • Any follow-up and re-evaluation are included in the initial reimbursement.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Scaling in presence of generalized moderate or severe gingival inflammation – full mouth, after oral evaluation	D4346	\$57.89	\$47.89	\$10.00		The removal of plaque, calculus, and stains from supra- and sub-gingival tooth surfaces when there is generalized moderate or severe gingival inflammation in the absence of periodontitis. It is indicated for patients who have swollen, inflamed gingiva, generalized suprabony pockets, and moderate to severe bleeding on probing. Should not be reported in conjunction with prophylaxis, scaling and root planing, or debridement procedures. Frequency: Two of D1110, D4346 per 12 months per patient. Not reimbursed when billed on the same date of service as D1110, D4341, D4342, D4355, D4910. - Any follow-up and re-evaluation are included in the initial reimbursement.
Full mouth debridement to enable a comprehensive periodontal evaluation and diagnosis on a subsequent visit	D4355	\$100.04	\$90.04	\$10.00		One of (D4335) per 36 months per patient. D0150, D0160, D0180 D1110 is not reimbursable when provided on the same day of service as D4355. D4355 is not reimbursable if patient record indicates D1110 or D4910 have been provided in the previous 12 month period. Other D4000 series codes are not reimbursable when provided on the same date of service as D4355.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Periodontal maintenance	D4910	\$125.91	\$125.91	\$0.00		This procedure is instituted following periodontal therapy and continues at varying intervals, determined by the clinical evaluation of the dentist, for the life of the dentition or any implant replacements. It includes removal of the bacterial plaque and calculus from supragingival and subgingival regions, site specific scaling and root planing where indicated and polishing the teeth. If new or recurring periodontal disease appears, additional diagnostic and treatment procedures must be considered. Frequency: • Up to four times per fiscal year per patient. • Cannot be charged on the same date as D4346. • Cannot be charged within the first three months following active periodontal treatment.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Complete denture - maxillary	D5110	\$931.41	\$851.41	\$80.00		Reimbursement made upon delivery of a complete maxillary denture to the patient. D5110 or D5120 cannot be used to report an immediate denture, D5130 or D5140. Routine follow-up adjustments/relines within 6 months are to be anticipated and are included in the initial reimbursement. A complete denture is made after teeth have been removed and the gum and bone tissues have healed - or to replace an existing denture. This can vary greatly depending upon patient, oral health, overall health, and other confounding factors. Frequency: Program will only pay for one per every 60 months - documentation that existing prosthesis cannot be made serviceable must be maintained.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Complete denture - mandibular	D5120	\$932.94	\$852.94	\$80.00		Reimbursement made upon delivery of a complete mandibular denture to the patient. D5110 or D5120 cannot be used to report an immediate denture, D5130, D5140. Routine follow-up adjustments/relines within 6 months are to be anticipated and are included in the initial reimbursement. A complete denture is made after teeth have been removed and the gum and bone tissues have healed - or to replace an existing denture. This can vary greatly depending upon patient, oral health, overall health, and other confounding factors. Frequency: Program will only pay for one per every 60 months - documentation that existing prosthesis cannot be made serviceable must be maintained.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Immediate denture – maxillary	D5130	\$931.41	\$851.41	\$80.00		Reimbursement made upon delivery of an immediate maxillary denture to the patient. Routine follow-up adjustments/soft tissue condition relines within 6 months are to be anticipated and are included in the initial reimbursement. An immediate denture is made prior to teeth being extracted and is inserted same day of extraction of remaining natural teeth. Frequency: D5130 can be reimbursed only once per lifetime per patient. Complete denture, D5110, may be considered 60 months after immediate denture was reimbursed. Documentation that existing prosthesis cannot be made serviceable must be maintained. Immediate Denture Form must be on file.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Immediate denture – mandibular	D5140	\$932.94	\$852.94	\$80.00		Reimbursement made upon delivery of an immediate mandibular denture to the patient. Routine follow-up adjustments/soft tissue condition relines within 6 months are to be anticipated and are included in the initial reimbursement. An immediate denture is made prior to teeth being extracted and is inserted same day of extraction of remaining natural teeth. Frequency: D5140 can be reimbursed only once per lifetime per patient. Complete dentures, D5120, may be considered 60 months after immediate denture was reimbursed – documentation that existing prosthesis cannot be made serviceable must be maintained. Immediate Denture Form must be on file.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Maxillary partial denture - resin base (including retentive/clasp materials, rests, and teeth)	D5211	\$646.83	\$586.83	\$60.00		Reimbursement made upon delivery of a complete partial maxillary denture to the patient. D5211 and D5212 are considered definitive treatments. Routine follow-up adjustments or relines within 6 months are to be anticipated and are included in the initial reimbursement. A partial resin base denture can be made right <u>after</u> having teeth extracted (healing from only a few teeth is not as extensive as healing from multiple). A partial resin base denture can also be made before having teeth extracted if the teeth being removed are in the front or necessary healing will be minimal. Several impressions and "try-in" appointments may be necessary and are included in the cost. Frequency: Program will only pay for one resin maxillary per every 36 months - documentation that existing prosthesis cannot be made serviceable must be maintained.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Mandibular partial denture - resin base (including retentive/clasp materials, rests, and teeth)	D5212	\$646.83	\$586.83	\$60.00		Reimbursement made upon delivery of a complete partial mandibular denture to the patient. D5211 and D5212 are considered definitive treatment. Routine follow-up adjustments/relines within 6 months are to be anticipated and are included in the initial reimbursement. A partial resin base denture can be made right <u>after</u> having teeth extracted (healing from only a few teeth is not as extensive as healing from multiple). A partial resin base denture can also be made before having teeth extracted if the teeth being removed are in the front or necessary healing will be minimal. Several impressions and "try-in" appointments may be necessary and are included in the cost. Frequency: Program will only pay for one every 36 months - documentation that existing prosthesis cannot be made serviceable must be maintained.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Maxillary partial denture – cast metal framework with resin denture bases (including retentive/clasp materials, rests and teeth)	D5213	\$900.48	\$840.48	\$60.00		Reimbursement made upon delivery of a complete partial maxillary denture to the patient. D5213 and D5214 are considered definitive treatment. Routine follow-up adjustments or relines within 6 months are to be anticipated and are included in the initial reimbursement. A partial cast metal base can also be made right after having teeth extracted (healing from only a few teeth is not as extensive as healing from multiple). A partial cast metal base denture can be made before having teeth extracted if the teeth being removed are in the front or necessary healing will be minimal. Several impressions and “try-in” appointments may be necessary and are included in the cost. Frequency: Program will only pay for one maxillary per every 60 months - documentation that existing prosthesis cannot be made serviceable must be maintained.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Mandibular partial denture – cast metal framework with resin denture bases (including retentive/clasp materials, rests and teeth)	D5214	\$900.48	\$840.48	\$60.00		Reimbursement made upon delivery of a complete partial mandibular denture to the patient. D5213 and D5214 are considered definitive treatment. Routine follow-up adjustments or relines within 6 months are to be anticipated and are included in the initial reimbursement. A partial cast metal base can be made right after having teeth extracted (healing from only a few teeth is not as extensive as healing from multiple). A partial cast metal base denture can also be made before having teeth extracted if the teeth being removed are in the front or necessary healing will be minimal. Several impressions and “try-in” appointments may be necessary and are included in the cost. Frequency: Program will only pay for one mandibular per every 60 months - documentation that existing prosthesis cannot be made serviceable must be maintained.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Immediate maxillary partial denture – resin base (including retentive/clasp materials, rests and teeth)	D5221	\$646.83	\$586.83	\$60.00		Reimbursement made upon delivery of an immediate partial maxillary denture to the patient. D5221 can be reimbursed only once per lifetime per patient and must be on the same date of service as the extraction. Routine follow-up adjustments or relines within 6 months is to be anticipated and are included in the initial reimbursement. An immediate partial resin base denture can be made before having teeth extracted if the teeth being removed are in the front or necessary healing will be minimal. Several impressions and “try-in” appointments may be necessary and are included in the cost. Frequency: A maxillary partial denture may be considered 36 months after immediate partial denture was reimbursed. Documentation that existing prosthesis cannot be made serviceable must be maintained. Immediate Denture Form must be on file.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Immediate mandibular partial denture – resin base (including retentive/clasp materials, rests and teeth)	D5222	\$646.83	\$586.83	\$60.00		Reimbursement made upon delivery of an immediate partial mandibular denture to the patient. D5222 can be reimbursed only once per lifetime per patient and must be on the same date of service as the extraction. Routine follow-up adjustments or relines within 6 months is to be anticipated and are included in the initial reimbursement. An immediate partial resin base denture can be made before having teeth extracted if the teeth being removed are in the front or necessary healing will be minimal. Several impressions and “try-in” appointments may be necessary and are included in the cost. Frequency: A mandibular partial denture may be considered 36 months after immediate partial denture was reimbursed. Documentation that existing prosthesis cannot be made serviceable must be maintained. Immediate Denture Form must be on file.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Immediate maxillary partial denture – cast metal framework with resin denture bases (including retentive/clasp materials, rests and teeth)	D5223	\$900.48	\$840.48	\$60.00		Reimbursement made upon delivery of an immediate partial maxillary denture to the patient. D5223 can be reimbursed only once per lifetime per patient and must be on the same date of service as the extraction. Routine follow-up adjustments or relines within 6 months is to be anticipated and are included in the initial reimbursement. An immediate partial cast metal framework with resin base denture can be made before having teeth extracted if the teeth being removed are in the front or necessary healing will be minimal. Several impressions and “try-in” appointments may be necessary and are included in the cost. Frequency: A maxillary partial denture may be considered 60 months after immediate partial denture was reimbursed. Documentation that existing prosthesis cannot be made serviceable must be maintained. Immediate Denture Form must be on file.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Immediate mandibular partial denture – cast metal framework with resin denture bases (including retentive/clasp materials, rests and teeth)	D5224	\$900.48	\$840.48	\$60.00		Reimbursement made upon delivery of an immediate partial mandibular denture to the patient. D5224 can be reimbursed only once per lifetime per patient and must be on the same date of service as the extraction. Routine follow-up adjustments or relines within 6 months are to be anticipated and are included in the initial reimbursement. An immediate partial cast metal framework with resin base denture can be made before having teeth extracted if the teeth being removed are in the front or necessary healing will be minimal. Several impressions and “try-in” appointments may be necessary and are included in the cost. Frequency: A mandibular partial denture may be considered 60 months after immediate partial denture was reimbursed. Documentation that existing prosthesis cannot be made serviceable must be maintained. Immediate Denture Form must be on file.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Maxillary partial denture – flexible base (including retentive/clasp materials, rests, and teeth)	D5225	\$798.83	\$738.83	\$60.00		Reimbursement made upon delivery of a partial maxillary denture to the patient. D5225 and D5226 are considered definitive treatment. Routine follow-up adjustments or relines within 6 months are to be anticipated and are included in the initial reimbursement. A partial flexible base can be made right after having teeth extracted (healing from only a few teeth is not as extensive as healing from multiple). A partial flexible base denture can also be made before having teeth extracted if the teeth being removed are in the front or necessary healing will be minimal. Several impressions and “try-in” appointments may be necessary and are included in the cost. Frequency: Program will only pay for one maxillary per every 36 months - documentation that existing prosthesis cannot be made serviceable must be maintained.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Mandibular partial denture – flexible base (including retentive/clasp materials, rests, and teeth)	D5226	\$798.83	\$738.83	\$60.00		Reimbursement made upon delivery of a partial mandibular denture to the patient. D5225 and D5226 are considered definitive treatment. Routine follow-up adjustments or relines within 6 months are to be anticipated and are included in the initial reimbursement. A partial flexible base can be made right after having teeth extracted (healing from only a few teeth is not as extensive as healing from multiple). A partial flexible base denture can also be made before having teeth extracted if the teeth being removed are in the front or necessary healing will be minimal. Several impressions and “try-in” appointments may be necessary and are included in the cost. Frequency: Program will only pay for one mandibular per every 36 months - documentation that existing prosthesis cannot be made serviceable must be maintained.
Adjust Complete Denture - maxillary	D5410	\$54.41	\$44.41	\$10.00		Adjust complete maxillary denture. Frequency: two of D5410 per 12 months per client. Cannot be charged on a denture provided in last six months. Cannot be charged in addition to a rebase or reline in a 12 month period.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Adjust Complete Denture - mandibular	D5411	\$54.41	\$44.41	\$10.00		Adjust complete maxillary denture. Frequency: two of D5411 per 12 months per client. Cannot be charged on a denture provided in last six months. Cannot be charged in addition to a rebase or reline in a 12 month period.
Adjust Partial Denture - maxillary	D5421	\$54.41	44.41	\$10.00		Adjust complete maxillary denture. Frequency: two of D5421 per 12 months per client. Cannot be charged on a denture provided in last six months. Cannot be charged in addition to a rebase or reline in a 12 month period.
Adjust Partial Denture - mandibular	D5422	\$54.41	\$44.41	\$10.00		Adjust complete maxillary denture. Frequency: two of D5422 per 12 months per client. Cannot be charged on a denture provided in last six months. Cannot be charged in addition to a rebase or reline in a 12 month period.
Repair broken complete denture base, mandibular	D5511	\$131.84	\$121.84	\$10.00		Repair broken complete mandibular denture base. Frequency: Two of D5511 per 12 months per patient.
Repair broken complete denture base, maxillary	D5512	\$131.84	\$121.84	\$10.00		Repair broken complete maxillary denture base. Frequency: Two of D5512 per 12 months per patient.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Replace missing or broken teeth - complete denture - per tooth	D5520	\$98.85	\$88.85	\$10.00	Teeth 1 - 32	Replacement/repair of missing or broken teeth. Teeth 1 – 32 and must report tooth number.
Repair resin partial denture base, mandibular	D5611	\$99.55	\$89.55	\$10.00		Repair resin partial mandibular denture base. Frequency: Two D5611 per 12 months per patient.
Repair resin partial denture base, maxillary	D5612	\$99.55	\$89.55	\$10.00		Repair resin partial maxillary denture base. Frequency: Two D5612 per 12 months per patient.
Repair cast partial framework, mandibular	D5621	\$129.27	\$119.27	\$10.00		Repair cast partial mandibular framework. Frequency: Two of D5621 per 12 months per patient.
Repair cast partial framework, maxillary	D5622	\$129.27	\$119.27	\$10.00		Repair cast partial maxillary framework. Frequency: Two of D5622 per 12 months per patient.
Repair or replace broken retentive/clasp materials – per tooth	D5630	\$139.66	\$129.66	\$10.00	Teeth 1 - 32	Repair of broken clasp on partial denture base – per tooth. Teeth 1 – 32, report tooth number(s). Frequency: One of D5630 per 12 months per patient per tooth.
Replace missing or broken teeth-per tooth	D5640	\$100.04	\$90.04	\$10.00	Teeth 1 - 32	Repair/replacement of missing tooth. Teeth 1 – 32, report tooth number(s). Frequency: One of D5640 per 12 months per patient per tooth.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Add tooth to existing partial denture – per tooth	D5650	\$89.86	\$79.86	\$10.00	Teeth 1 - 32	Adding tooth to partial denture base. Frequency: One of D5650 per 12 months per patient per tooth. Documentation may be requested when charged on partial delivered in last 12 months. Teeth 1 – 32, report tooth number(s).
Add clasp to existing partial denture – per tooth	D5660	\$145.08	\$135.08	\$10.00	Teeth 1 - 32	Adding clasp to partial denture base – per tooth. Frequency: One of D5660 per 12 months per patient per tooth. Documentation may be requested when charged on partial delivered in last 12 months. Teeth 1 – 32, report tooth number(s).
Rebase complete maxillary denture	D5710	\$307.15	\$282.15	\$25.00		Frequency: One time per 12 months. Completed at laboratory. Cannot be charged on denture provided in the last 6 months. Cannot be charged in addition to a reline in a 12 month period.
Rebase complete mandibular denture	D5711	\$308.35	\$283.35	\$25.00		Frequency: One time per 12 months. Completed at laboratory. Cannot be charged on denture provided in the last 6 months. Cannot be charged in addition to a reline in a 12 month period.
Rebase maxillary partial denture	D5720	\$295.74	\$270.74	\$25.00		Frequency: One time per 12 months. Completed at laboratory. Cannot be charged on denture provided in the last 6 months. Cannot be charged in addition to a reline in a 12 month period.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Rebase mandibular partial denture	D5721	\$295.74	\$270.74	\$25.00		Frequency: One time per 12 months. Completed at laboratory. Cannot be charged on denture provided in the last 6 months. Cannot be charged in addition to a reline in a 12 month period.
Reline complete maxillary denture (chairside/direct)	D5730	\$190.08	\$180.08	\$10.00		Frequency: One time per 12 months. Cannot be charged on denture provided in the last 6 months. Cannot be charged in addition to a rebase in a 12 month period.
Reline complete mandibular denture (chairside/direct)	D5731	\$190.08	\$180.08	\$10.00		Frequency: One time per 12 months. Cannot be charged on denture provided in the last 6 months. Cannot be charged in addition to a rebase in a 12 month period.
Reline maxillary partial denture (chairside/direct)	D5740	\$187.69	\$177.69	\$10.00		Frequency: One time per 12 months. Cannot be charged on denture provided in the last 6 months. Cannot be charged in addition to a rebase in a 12 month period.
Reline mandibular partial denture (chairside/direct)	D5741	\$189.49	\$179.49	\$10.00		Frequency: One time per 12 months. Cannot be charged on denture provided in the last 6 months. Cannot be charged in addition to a rebase in a 12 month period.
Reline complete maxillary denture (laboratory/in direct)	D5750	\$253.13	\$228.13	\$25.00		Frequency: One time per 12 months. Cannot be charged on denture provided in the last 6 months. Cannot be charged in addition to a rebase in a 12 month period.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Reline complete mandibular denture (laboratory/in direct)	D5751	\$254.31	\$229.31	\$25.00		Frequency: One time per 12 months. Cannot be charged on denture provided in the last 6 months. Cannot be charged in addition to a rebase in a 12 month period.
Reline maxillary partial denture (laboratory/in direct)	D5760	\$251.33	\$226.33	\$25.00		Frequency: One time per 12 months. Cannot be charged on denture provided in the last 6 months. Cannot be charged in addition to a rebase in a 12 month period.
Reline mandibular partial denture (laboratory/in direct)	D5761	\$251.33	\$226.33	\$25.00		Frequency: One time per 12 months. Cannot be charged on denture provided in the last 6 months. Cannot be charged in addition to a rebase in a 12 month period.
Extraction, erupted tooth or exposed root (elevation and/or forceps removal)	D7140	\$119.07	\$109.07	\$10.00	Teeth 1 - 32	Includes removal of tooth structure, minor smoothing of socket bone, and closure as necessary. Frequency: One of D7140 per lifetime per patient per tooth. Teeth 1 – 32.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Extraction, erupted tooth requiring removal of bone and/or sectioning of tooth, and including elevation of mucoperiosteal flap if indicated	D7210	\$184.54	\$174.54	\$10.00	Teeth 1 - 32	Includes related cutting of gingiva and bone, removal of tooth structure, minor smoothing of socket bone and closure. Frequency: One of D7210 per lifetime per patient per tooth. Teeth 1 - 32
Removal of impacted tooth-soft tissue	D7220	\$220.66	\$200.66	\$20.00	Teeth 1 - 32	Occlusal surface of tooth covered by soft tissue; requires mucoperiosteal flap elevation. Teeth 1-32. Frequency: One of D7220 per lifetime per patient per tooth.
Removal of impacted tooth-partially bony	D7230	\$272.40	\$252.40	\$20.00	Teeth 1 - 32	Part of crown covered by bone; requires mucoperiosteal flap elevation and bone removal. Teeth 1-32. Frequency: One of D7230 per lifetime per patient per tooth
Removal of impacted tooth-completely bony	D7240	\$316.18	\$296.18	\$20.00	Teeth 1 - 32	Most or all of crown covered by bone; requires mucoperiosteal flap elevation and bone removal. Teeth 1-32. Frequency: One of D7240 per lifetime per patient per tooth.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Removal of impacted tooth-completely boney, with unusual surgical complications	D7241	\$415.64	\$395.64	\$20.00	Teeth 1 - 32	Most or all of crown covered by bone; unusually difficult or complicated due to factors such as nerve dissection required, separate closure of maxillary sinus required or aberrant tooth position. Teeth 1-32. Frequency: One of D7241 per lifetime per patient per tooth.
Removal of residual tooth roots (cutting procedure)	D7250	\$194.64	\$184.64	\$10.00	Teeth 1 - 32	Includes cutting of soft tissue and bone, removal of tooth structure, and closure. Cannot be charged for removal of broken off roots for recently extracted tooth. Teeth 1 – 32 Frequency: One of D7250 per lifetime per patient per tooth. Will not be paid to the dentists or group that removed the tooth.
Primary closure of a sinus perforation	D7261	\$485.35	\$475.35	\$10.00		Subsequent to surgical removal of tooth, exposure of sinus requiring repair, or immediate closure of oroantral or oralnasal communication in absence of fistulous tract. Narrative of medical necessity may be required and if the sinus perforation was caused by a current grantee or provider of the program.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Incisional biopsy of oral tissue - hard (bone, tooth)	D7285	\$196.67	\$186.67	\$10.00		For partial removal of lesion only. This procedure involves biopsy of osseous or intra-osseous lesions (example cyst, tumor) and is not used for apicectomy/periradicular surgery. This procedure does not entail an excision. Only covered if there is a suspicious lesion. Must have a pathology report in file.
Incisional biopsy of oral tissue-soft	D7286	\$158.88	\$148.88	\$10.00		For partial removal of lesion . This procedure is not used at the same time as codes for apicoectomy/periradicular curettage. This procedure does not entail an excision. Treatment notes must include documentation and proof that biopsy was sent for evaluation. Only covered if there is a suspicious lesion.
Alveoloplasty in conjunction with extractions - four or more teeth or tooth spaces, per quadrant	D7310	\$148.69	\$138.69	\$10.00	Per Quadrant LL, LR, UL, UR	The alveoloplasty is distinct (separate procedure) from extractions. Usually in preparation for prosthesis or other treatments such as radiation therapy and transplant surgery. Frequency: One of D7310 or D7311 per lifetime per patient per quadrant. Minimum of 4 extractions in the affected quadrant.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Alveoloplasty in conjunction with extractions - one to three teeth or tooth spaces, per quadrant	D7311	\$148.69	\$138.69	\$10.00	Per Quadrant LL, LR, UL, UR	The alveoloplasty is distinct (separate procedure) from extractions. Usually in preparation for a prosthesis or other treatments such as radiation therapy and transplant surgery. Frequency: One of D7311 or D7310 per lifetime per patient per quadrant. Maximum of 3 extractions in the affected quadrant.
Alveoloplasty not in conjunction with extractions - four or more teeth or tooth spaces, per quadrant	D7320	\$214.114	\$204.11	\$10.00	Per Quadrant LL, LR, UL, UR	No extractions performed in an edentulous area. See D7310 if teeth are being extracted concurrently with the alveoloplasty. Usually in preparation for prosthesis or other treatments such as radiation therapy and transplant surgery. Frequency: One of D7320 or D7321 per lifetime per patient per quadrant.
Alveoloplasty not in conjunction with extractions - one to three teeth or tooth spaces, per quadrant	D7321	\$214.11	\$204.11	\$10.00	Per Quadrant LL, LR, UL, UR	No extractions performed in an edentulous area. See D7311 if teeth are being extracted concurrently with the alveoloplasty. Usually in preparation for prosthesis or other treatments such as radiation therapy and transplant surgery. Frequency: One of D7320 or D7321 per lifetime per patient per quadrant.
Excision of benign Lesion up to 1.25 cm	D7410	200.90	190.90	\$10.00		Must have a pathology report in file.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Removal of benign nonodontogenic cyst or tumor-lesion diameter up to 1.25 cm	D7460	\$253.47	\$243.47	\$10.00		Must have a pathology report in file.
Removal of lateral exostosis (maxilla or mandible)	D7471	\$310.17	\$300.17	\$10.00	Per Arch LA, UA	Limited to the removal of exostosis, including the removal of tori, osseous tuberosities, and other osseous protuberances, when the mass prevents the seating of denture and does not allow denture seal.
Removal of torus palatinus	D7472	\$364.79	\$354.79	\$10.00	Per Quadrant LL, LR, UL, UR	Limited to the removal of exostosis, including the removal of tori, osseous tuberosities, and other osseous protuberances, when the mass prevents the seating of denture and does not allow denture seal.
Removal of torus mandibularis	D7473	\$355.79	\$345.79	\$10.00	Per Quadrant LL, LR, UL, UR	Limited to the removal of exostosis, including the removal of tori, osseous tuberosities, and other osseous protuberances, when the mass prevents the seating of denture and does not allow denture seal.
Incision & drainage of abscess - intraoral soft tissue	D7510	\$119.26	\$109.26	\$10.00	Teeth 1 - 32	Incision through mucosa, including periodontal origins. One of D7510 per lifetime per patient per tooth.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Palliative treatment of dental pain – per visit	D9110	\$82.04	\$57.04	\$25.00		Treatment that relieves pain but is not curative; services provided do not have distinct procedure codes. Not allowed with any other services other than radiographs. Cannot be billed when the only other service is writing a prescription.
Evaluation for moderate sedation, deep sedation or general anesthesia	D9219	\$43.83	\$43.83	\$0.00		One of D9219 per 12 months per grantee per patient.
Administration of deep sedation/general anesthesia - first 15 minute increment, or any portion thereof With or without co-administration of nitrous oxide	D9222	\$124.76	\$114.76	\$10.00		One of D9222 per 1 day per patient.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Administration of deep sedation/general anesthesia - each subsequent 15 minute increment, or any portion thereof	D9223	\$110.09	\$100.09	\$10.00		Not allowed with D9243
Administration of moderate sedation - intravenous - first 15 minute increment, or any portion thereof When moderate sedation is achieved by administration and titration of drug(s) intravenously . With or without co-administration of nitrous oxide.	D9239	\$124.76	\$114.76	\$10.00		Anesthesia time begins when the doctor administering the anesthetic agent initiates the appropriate anesthesia and non-invasive monitoring protocol and remains in continuous attendance of the patient. Anesthesia services are considered completed when the patient may be safely left under the observation of trained personnel and the doctor may safely leave the room. The level of anesthesia is determined by the provider's documentation of the anesthetic effects upon the central nervous system. One of D9239 per 1 day per patient.

Procedure Description	Alpha-numeric Code	Max Allowable Fee	Program Payment	Max Patient Co-Pay	Teeth or Quadrant Covered	PROGRAM GUIDELINES
Administration of moderate sedation - intravenous - each subsequent 15 minute increment, or any portion thereof	D9243	\$110.09	\$100.09	\$10.00		Thirteen of D9243 per 1 day per patient. Not allowed with D9223

EXPLANATION OF RESTORATIONS		
Location	Number of Surfaces	Characteristics
Anterior - Mesial, Distal, Incisal, Lingual, or Facial (or Labial)	1	Placed on one of the five surface classifications. .
	2	Placed, without interruption, on two of the surface classifications.
	3	Placed, without interruption, on three of the surface classifications.
	4 or more	Placed, without interruption, on four or more of the surface classifications.
Posterior – Mesial, Distal, Occlusal, Lingual, or Buccal	1	Placed on one of the five surface classifications.
	2	Placed, without interruption, on two of the surface classifications.
	3	Placed, without interruption, on three of the surface classifications.
	4 or more	Placed, without interruption, on four or more of the surface classifications.

NOTE: Tooth surfaces are reported using the letters in the following table.

Surface	Code
Buccal	B
Distal	D
Facial (or Labial)	F
Incisal	I

Surface	Code
Lingual	L
Mesial	M
Occlusal	O

8.970 PROVIDER STABILIZATION FUND

8.970.1 GENERAL DESCRIPTION

8.970.1.A In accordance with Colorado Senate Bill (SB) 25-290 Stabilization Payments for Safety Net Providers, the Provider Stabilization Fund was created to make stabilization payments to eligible safety net providers. The bill directed the state treasury to establish a Provider Stabilization Fund to collect the monies to be used for payments and set forth how the funds will be allocated and designated the Department of Health Care Policy & Financing (the Department) as the administrator of the payments.

8.970.1.B The Provider Stabilization Fund provides an allocation of monies to safety net providers that serve individuals who are considered low-income and uninsured. Monies shall be allocated based on the number of eligible patients in an amount proportional to the total number of eligible patients served by all safety net providers who qualify for monies from this fund.

8.970.2 DEFINITIONS

8.970.2.A Advisory Board means The Provider stabilization fund advisory board created pursuant to C.R.S. § 25.5-3-605,

8.970.2.B. Children's Basic Health Plan also known as Child Health Plan Plus (CHP+) means the program as specified in Article 8 of Title 25.5, C.R.S.

8.970.2.C. Comprehensive Behavioral Health Provider has the same meaning as defined at C.R.S. § 27-50-101(11).

8.970.2.D. Eligible Patient means a low-income, uninsured individual who is a patient receiving medical services from a Qualified Provider:

1. Whose annual household income is at or below two hundred percent (200%) of the Federal Poverty Guideline (FPG);
2. Who is not enrolled in the Medical Assistance Program, the Children's Basic Health Plan, Medicare or any other governmental health care coverage such as through Social Security, the Veterans Administration, Military Dependency (TRICARE or CHAMPUS), or the United States Public Health Service; and
3. For whom there is no Third Party Payer paying or reimbursing the safety net provider for all or a portion of the amount charged for the services provided to the individual.

8.970.2.E. Eligible Safety Net Provider - A Safety Net Provider who is identified by the Department to receive funding from the Provider Stabilization Fund.

- 8.970.2.G. Monies means funds appropriated, transferred, or credited to the Provider Stabilization Fund created in the State Treasury consisting of:
1. Money credited to the fund as a loan from the unclaimed property trust fund pursuant to C.R.S. § 38-13-801(6) ;
 2. Money appropriated, transferred, or credited to the fund by the general assembly;
 3. Gifts, grants, or donations the Department may receive from public or private sources for the fund.
- 8.970.2.H. Outside Entity means a business or professional that is not classified as an employee of the provider or the Department and does not have a direct or indirect financial interest with the provider, but has auditing experience or experience working directly with the Medical Assistance Program or similar services or grants for Eligible Patients.
- 8.970.2.I. Primary Care means care that deals with the individual rather than an organ system or an abnormal physiology and provides an array of services covering the preventive, diagnostic, and therapeutic needs of patients, including referral and coordination of care to other services. For purposes of the Provider Stabilization Fund, primary care may include other services based on a patient's needs including dental, comprehensive behavioral health, and vision.
1. 8.970.2.J. Safety Net Provider means a provider as defined at C.R.S. § 25.5-3-602(8). A comprehensive Community Behavioral Health provider as defined at C.R.S. § 27-50-101(11);
 2. A Rural Health Clinic as defined at 42 U.S.C § 1395x(aa)(2);
 3. A Federally Qualified Health Center as defined at 42 U.S.C. § 1395x(aa)(4); or
 4. A health-care provider that is delivering primary care services and at least 50% of whose client caseload is individuals who are enrolled in Medicaid, Medicare, or the Children's Basic Health Plan or who are Low-Income, Uninsured Individuals.
- 8.970.2.K. Sliding Fee Schedule means a tiered co-payment system that determines the level of a patient's financial participation and guarantees that the patient financial participation is below usual and customary charges. Factors considered in establishing the tiered co-payment system are limited to financial status and the number of members in the patient's family unit.

8.970.2.L. Third Party Payments or Third Party Payer means any individual, entity or program with a legal obligation to pay for some or all health-related services rendered to a patient. Examples include the Medical Assistance Program; the Children's Basic Health Plan; Medicare; commercial, individual or employment-related health insurance; court-ordered health insurance (such as that required by non-custodial parents); workers' compensation; automobile insurance; and long-term care insurance.

8.970.2.M. Unduplicated User/Patient Count means the sum of patients who have had at least one Visit/Encounter with an eligible safety net provider during the applicable calendar year, but does not include the same patient more than once. The sum shall be calculated on a specific point-in-time occurring between the end of the applicable calendar year and prior to the submission of the application. Each patient shall be counted once under only one payment source designation (Third Party Payer or Eligible Patient). The patient's payment source designation shall be the payment source designation listed for the patient at the specific point-in-time in which the calculation is made. The sum shall not include:

1. Counting a patient more than once if the same patient returns for additional services (e.g., medical or dental) and/or products (e.g., pharmaceuticals) during the applicable calendar year;
2. Counting a patient more than once if the payment source designation changed during the applicable calendar year;
3. Persons who have only received services through an outreach event, community education program, nurse hotline, or other types of community-based events or programs and services were not recorded on an individual basis;
4. Persons who have only received services from large-scale efforts such as mass immunization programs, screening programs, and health fairs; or
5. Persons whose only contact with the provider is to receive Special Supplemental Nutrition Program for Women, Infants, and Children (WIC Program) counseling and vouchers are not users and the contact does not generate an encounter.

8.970.2.N. Visit/Encounter means an appointment with medical personnel (physicians, physician assistants, nurse practitioners, dentists, behavioral health workers, etc.) in which the patient received health related services and/or products (e.g., pharmaceuticals or radiology) and the appointment included primary care that is customarily billable by a safety net provider..

8.970.3 PROVIDER ELIGIBILITY

8.970.3.A. Safety Net Providers who provide Primary Care to Eligible Patients and who meet all the requirements established for the Provider Stabilization Fund as of the date the application form is submitted to the Department shall receive monies appropriated to the Provider Stabilization Fund. Specifically, the provider shall:

1. Meet one of the conditions of a Safety Net Provider as specified in Section 8.970.2.K.; and
2. Submit a completed application form according to stated guidelines as specified under Section 8.970.4.

8.970.4 APPLICATION

8.970.4.A. The application form shall be available to providers annually and posted for public access on the Department's website at least 30 calendar days prior to the application due date.

8.970.4.B. At a minimum, the application form shall require responses that:

1. Demonstrate how the provider meets the criteria of a Safety Net Provider as defined in Section 8.970.2.K.;
2. Provide an Unduplicated User/Patient Count covering the applicable calendar year which, at a minimum, includes the number of patients enrolled in the Medical Assistance Program and the Children's Basic Health Plan and the number of patients considered to be Eligible Patients; and
3. Provide certification that the Unduplicated User/Patient Count identified in Section 8.970.4.B.2 has been verified by an Outside Entity.

8.970.4.C. Providers must complete and provide an application annually. The application must be made in compliance with all specifications in the application form, including format, data and documentation. Applications must be submitted directly to the Department by the required deadline.

8.970.4.D. All providers who submit an application will be notified within 45 days of the application deadline if the provider has met or did not meet the requirements to be a Eligible Qualified Provider.

8.970.4.E. Safety Net Providers who are eligible for the Primary Care Fund are able to use their Primary Care Fund application for the Provider Stabilization Fund. Such providers must submit the Provider Stabilization Fund Attestation Form to the Department. The attestation form shall:

1. Be available to providers annually and posted for public access on the Department's website at least 30 calendar days prior to the application due date;
2. Provide attestation that the Safety Net Provider wishes to use the data from their Primary Care Fund application for the Provider Stabilization Fund application;
3. Provide Safety Net Provider contact information and be signed by a representative of the Safety Net Provider.

8.970.5 DISBURSEMENT

- 8.970.5.A. Eligible Safety Net Providers are determined on a state fiscal year basis and shall receive only those monies received by the Provider Stabilization Fund during that same state fiscal year. Monies disbursed shall include all monies defined in 8.970.2.G.
- 8.970.5.B. Payments shall be based on the number of Eligible Patients in each Eligible Safety Net Provider's Unduplicated User/Patient Count in an amount proportionate to the total number of Eligible Patients from all Eligible Safety Net Providers' Unduplicated User/Patient Counts.
- 8.970.5.C. The schedule for the disbursement of monies to all Eligible Safety Net Providers shall be dependent on the source and when the funds are available to the Department, with a schedule as follows:
1. Money received by the Department as a loan from the unclaimed property trust or money appropriated, credited, or transferred by the general assembly shall be distributed to eligible providers no later than March 31 in State Fiscal Year 2025-26 and by September 30 in following State Fiscal Years.
 2. Money received as a gift, grant, or donation shall be distributed in the first month of the quarter following the quarter the money was received. (Example: Money gifted, granted, or donated between July 1 and September 30 would be distributed by October 3. Money received as a gift, grant, or donation during State Fiscal Year 2025-26 Quarters One and Two (July 1, 2025 - December 31, 2025) would be distributed no later than March 31, 2026.

8.970.6 ADVISORY BOARD

- 8.970.6.A. Advisory Board function and duties
1. Collaborate with the Department to seek, accept and expend gifts, grants or donations from private or public sources.

2. Collaborate with the Department to annually allocate money appropriated by the general assembly to the Provider Stabilization Fund.
3. Assist the Department with the annual Provider Stabilization Fund report as defined in C.R.S. § 25.5-3-606.
4. Act as consultation to the Department on obtaining federal matching money to the funds in the Provider Stabilization fund.
5. Support the Department with the implementation of the Safety Net Stabilization fund program.

8.970.6.B. Advisory Board appointment details. The advisory board was created by 25.5-3-605 to support the Department with the implementation of the Provider Stabilization Fund. Board members shall be appointed by the Governor with initial appointments being made no later than August 1, 2025. The Advisory Board shall elect a Chair and Vice-Chair from the provider and consumer members.

8.970.6.C. Advisory Board Membership Tenure, Compensation and Frequency of meetings shall be the following:

1. Advisory Board members shall serve a three-year term.
2. Advisory Board members shall serve without compensation or expense reimbursement.
3. Advisory board meetings shall be held quarterly or as determined necessary by the Chair.

8.970.6.D. Sunset Provisions. The Provider Stabilization Fund Advisory Board will be repealed, effective September 1, 2031, subject to review under the Colorado "Sunset" law.

Editor's Notes

10 CCR 2505-10 has been divided into smaller sections for ease of use. Versions prior to 03/04/2007, Statements of Basis and Purpose, and rule history are located in the first section, 10 CCR 2505-10. Prior versions can be accessed from the All Versions list on the rule's current version page. To view versions effective on or after 03/04/2007, select the desired section of the rule, for example 10 CCR 2505-10 8.100, or 10 CCR 2505-10 8.500.

History

[For history of this section, see Editor's Notes in the first section, 10 CCR 2505-10]